

R3 Recovery Services

618 Columbia Ave, Lancaster, Pennsylvania, US, 17603
(717) 349-9924

Date: 06/17/2026

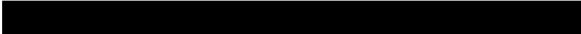
CLIENT PROFILE



Code Status: NA



Gender: Male
Pronoun(s): He/him/his
Birth Date: 12/31/1985



Primary Clinician: Marleigh Johnson
Status: Active
Admission Date: 04/23/2026
Discharge Date: 04/23/2026
Medical Provider:
Level Of Care: GOP
Treatment:

MEDICATIONS

Medication	Medication Type	Dosage	Instructions	Status	Start Date	End Date
Prozac	Home	20 mg capsule by mouth	Take 20 mg by mouth	Active	04/23/2026	
Vivitrol	Home	380 mg suspension, ER, reconstituted intramuscular	Take 380 mg intramuscular	Active	04/23/2026	

PRN MEDICATIONS

Medication	Medication Type	Dosage	Instructions	Status	Start Date	End Date
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ALLERGIES

-- No Allergies --

DIAGNOSIS

Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F10.20] Alcohol use disorder, Severe	Primary	Caleb Madison	04/23/2026	--

VITALS

-- No Vitals --

INSURANCE

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care	[REDACTED]	Self	None Specified	771216145812	Primary		Active

AUTHORIZATION

-- No Authorizations --

PRIVATE PAY

-- No Private Pay --

SUBSTANCE USE HISTORY

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Alcohol	0	Drinking	Unknown I would binge drink.	02/14/2026	20	9	buy it.	Binge

TESTING ANALYSIS

-- No Testing Analysis --

CLINICAL OPIATE WITHDRAWAL SCALE

-- No COWS --

CLINICAL INSTITUTE WITHDRAWAL ASSESSMENT

-- No CIWAs --

CLINICAL INSTITUTE WITHDRAWAL ASSESSMENT - BENZODIAZEPINES

-- No CIWA-Bs --

SHIFT NOTES

-- No Notes --

SHIFT ROUNDS

-- No Shift Rounds --

DOCTORS ORDERS

-- No Doctors Orders --

PROGRESS NOTES

-- No Progress Notes --

INDIVIDUAL APPOINTMENTS

04/24/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Canceled	3:00PM	4:00PM
Facility							
R3 Recovery Services							

Assessment Note:
Member will start next week!

Data:

Assessment:

Plan:

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

- ☒ WNL(within normal limit)
- ☐ Clean
- ☐ Unshaved
- ☐ Odorous
- ☐ Disheveled
- ☐ Other

Affect

- ☒ WNL(within normal limit)
- ☐ Appropriate
- ☐ Inappropriate
- ☐ Labile
- ☐ Blunted
- ☐ Flat
- ☐ Restricted
- ☐ Expansive
- ☐ Euphoric
- ☐ Anxious
- ☐ Angry
- ☐ Other

Hallucinations

- ☒ WNL(within normal limit)
- ☐ Visual
- ☐ Auditory
- ☐ Tactile
- ☐ Olfactory
- ☐ Command
- ☐ Denied
- ☐ Other

Movement

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Tense
- ☐ Fidgety
- ☐ Hyperactive
- ☐ Psychomotor Retardation
- ☐ Psychomotor Agitation
- ☐ Pacing
- ☐ Hand Wringing
- ☐ Tics
- ☐ Other

Mood

- ☒ WNL(within normal limit)
- ☐ Anxious
- ☐ Confused
- ☐ Depressed
- ☐ Labile
- ☐ Fearful
- ☐ Euphoric
- ☐ Angry
- ☐ Dysthymic
- ☐ Irritated
- ☐ Euthymic
- ☐ Normal
- ☐ Other

Speech

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Soft
- ☐ Loud
- ☐ Pressured
- ☐ Rapid
- ☐ Slowed
- ☐ Slurred
- ☐ Incoherent

Mental Status Examination

Impulse Control

☒ WNL(within normal limit)

☐ Good

☐ Limited

☐ Poor

Insight

☒ WNL(within normal limit)

☐ Good

☐ Limited

☐ Poor

Intelligence

☒ WNL(within normal limit)

☐ Above Average

☐ Average

☐ Below Average

Oriented

☒ WNL(within normal limit)

☐ Person

☐ Place

☐ Time

☐ Situation

Other

☒ WNL(within normal limit)

☐ Obsessions

☐ Compulsions

☐ Depersonalization

☐ DeRealism

☐ Denied

Memory

☒ WNL(within normal limit)

☐ Intact

☐ Immediate Impaired

☐ Short Term Impaired

☐ Long Term Impaired

Suicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Homicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Eye Contact

☒ WNL(within normal limit)

☐ Good

☐ Avoidant

☐ Fleeting

☐ Stare

Facial Expression

☒ WNL(within normal limit)

☐ Amiable

☐ Sad

☐ Happy

☐ Angry

☐ Worried

☐ Suspicious

Thought Process

☒ WNL(within normal limit)

☐ Intact

☐ Flight

☐ Blocked

☐ Circumstantial

☐ Loose

☐ Other

☐ Incoherent

☐ Tangential

Delusions

☒ WNL(within normal limit)

☐ Grandiose

☐ Denied

☐ Persecutory

☐ Other

☐ Somatic

☐ Paranoia

☐ Ideas

Demeanor

☒ WNL(within normal limit)

☐ Cooperative

☐ Friendly

☐ Irritable

☐ Uncooperative

☐ Worried

☐ Evasive

☐ Guarded

☐ Boastful

☐ Dramatic

☐ Resistive

☐ Demanding

☐ Seductive

☐ Defensive

☐ Hostile

☐ Arrogant

Mental Status Examination

☐ Apathetic ☐ Other

Attention/Concentration

☒ WNL(within normal limit) ☐ Satisfactory ☐ Poor ☐ Easily Distracted ☐ Other

Judgement

☒ WNL(within normal limit) ☐ Good ☐ Limited ☐ Poor

Reviewer

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
04/27/2026 7:35AM

05/01/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Canceled	3:00PM	4:00PM
Facility							
R3 Recovery Services							

Treatment Objectives:
Team meeting

Assessment Note:
Data: canceled for team meeting working to reschedule

Assessment:

Plan:

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

☒ WNL(within normal limit) ☐ Clean ☐ Unshaved ☐ Odorous ☐ Disheveled ☐ Other

Affect

Mental Status Examination

☒ WNL(within normal limit)	☐ Appropriate	☐ Inappropriate	☐ Labile	☐ Blunted	☐ Flat
☐ Other	☐ Restricted	☐ Expansive	☐ Euphoric	☐ Anxious	☐ Angry

Hallucinations

☒ WNL(within normal limit)	☐ Visual	☐ Auditory	☐ Tactile	☐ Olfactory	☐ Command
	☐ Denied	☐ Other			

Movement

☒ WNL(within normal limit)	☐ Normal	☐ Tense	☐ Fidgety	☐ Hyperactive	☐ Psychomotor Retardation
☐ Psychomotor Agitation	☐ Pacing	☐ Hand Wringing	☐ Tics	☐ Other	

Mood

☒ WNL(within normal limit)	☐ Anxious	☐ Confused	☐ Depressed	☐ Labile	☐ Fearful
	☐ Euphoric	☐ Angry	☐ Dysthymic	☐ Irritated	☐ Euthymic
☐ Normal	☐ Other				

Speech

☒ WNL(within normal limit)	☐ Normal	☐ Soft	☐ Loud	☐ Pressured	☐ Rapid
	☐ Slowed	☐ Slurred	☐ Incoherent		

Impulse Control

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Insight

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Intelligence

☒ WNL(within normal limit)	☐ Above Average	☐ Average	☐ Below Average
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Oriented

☒ WNL(within normal limit)	☐ Person	☐ Place	☐ Time	☐ Situation
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Other

☒ WNL(within normal limit)	☐ Obsessions	☐ Compulsions	☐ Depersonalization	☐ DeRealism	☐ Denied
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Memory

☒ WNL(within normal limit)	☐ Intact	☐ Immediate Impaired	☐ Short Term Impaired	☐ Long Term Impaired
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Suicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Homicidal Ideation

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Mental Status Examination

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Eye Contact

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
	☐ Sad				

Thought Process

☒ WNL(within normal limit)	☐ Intact	☐ Blocked	☐ Loose	☐ Incoherent	☐ Tangential
	☐ Flight	☐ Circumstantial	☐ Other		

Delusions

☒ WNL(within normal limit)	☐ Grandiose	☐ Persecutory	☐ Somatic	☐ Paranoia	☐ Ideas
	☐ Denied	☐ Other			

Demeanor

☒ WNL(within normal limit)	☐ Cooperative	☐ Uncooperative	☐ Guarded	☐ Resistive	☐ Defensive
☐ Irritable	☐ Friendly	☐ Worried	☐ Boastful	☐ Demanding	☐ Hostile
☐ Apathetic	☐ Agitated	☐ Evasive	☐ Dramatic	☐ Seductive	☐ Arrogant
	☐ Other				

Attention/Concentration

☒ WNL(within normal limit)	☐ Satisfactory	☐ Poor	☐ Easily Distracted	☐ Other
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Judgement

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Staff [REDACTED]

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/01/2026 10:02AM

05/05/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	3:00PM	4:00PM
Facility							
R3 Recovery Services							

Treatment Objectives:

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Assessment Note:

Data: Client presented for session and was engaged in discussing his personal background, including factors contributing to his substance use history and current life circumstances. Session focused on gathering comprehensive psychosocial history and collaboratively developing a treatment plan. Client identified his primary goals as achieving stability across legal, financial, and housing domains, as well as maintaining long-term sobriety. He verbalized strong motivation for recovery, specifically identifying his family as a key source of motivation and support. Client discussed his desire to remain sober in order to improve relationships and establish long-term stability. Clinician and client explored how past stressors and decision-making patterns have impacted his current situation and informed treatment planning accordingly.

Assessment: Client presents as motivated and engaged in the treatment process, demonstrating insight into his substance use patterns and contributing life stressors. He shows readiness for change and commitment to recovery, particularly through his identification of family as a sustaining motivator. Development of a structured treatment plan highlights client's ability to participate in goal-setting and future-oriented thinking. Ongoing stressors, including legal and financial challenges, continue to pose risk factors; however, client's motivation and engagement support continued progress in treatment. Clinical presentation supports continued need for services to maintain stability and prevent relapse.

Plan: Continue treatment in alignment with established goals focused on maintaining sobriety, improving overall stability, and strengthening coping skills. Interventions will include relapse prevention strategies, exploration of triggers, and development of adaptive coping mechanisms. Clinician will support client in working toward identified goals, including improving decision-making related to finances and housing, as well as reinforcing motivation tied to family relationships. Future sessions will monitor progress toward treatment goals and adjust interventions as needed to support sustained recovery.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

- ☒ WNL(within normal limit)
- ☐ Clean
- ☐ Unshaved
- ☐ Odorous
- ☐ Disheveled
- ☐ Other

Affect

- ☒ WNL(within normal limit)
- ☐ Appropriate
- ☐ Inappropriate
- ☐ Labile
- ☐ Blunted
- ☐ Flat
- ☐ Restricted
- ☐ Expansive
- ☐ Euphoric
- ☐ Anxious
- ☐ Angry
- ☐ Other

Hallucinations

- ☒ WNL(within normal limit)
- ☐ Visual
- ☐ Auditory
- ☐ Tactile
- ☐ Olfactory
- ☐ Command
- ☐ Denied
- ☐ Other

Movement

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Tense
- ☐ Fidgety
- ☐ Hyperactive
- ☐ Psychomotor Retardation
- ☐ Psychomotor Agitation
- ☐ Pacing
- ☐ Hand Wringing
- ☐ Tics
- ☐ Other

Mood

- ☒ WNL(within normal limit)
- ☐ Anxious
- ☐ Confused
- ☐ Depressed
- ☐ Labile
- ☐ Fearful
- ☐ Euphoric
- ☐ Angry
- ☐ Dysthymic
- ☐ Irritated
- ☐ Euthymic
- ☐ Normal
- ☐ Other

Speech

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Soft
- ☐ Loud
- ☐ Pressured
- ☐ Rapid
- ☐ Slowed
- ☐ Slurred
- ☐ Incoherent

Impulse Control

- ☒ WNL(within normal limit)
- ☐ Good
- ☐ Limited
- ☐ Poor

Insight

Mental Status Examination

☒ WNL(within normal limit)

☐ Good

☐ Limited

☐ Poor

Intelligence

☒ WNL(within normal limit)

☐ Above Average

☐ Average

☐ Below Average

Oriented

☒ WNL(within normal limit)

☐ Person

☐ Place

☐ Time

☐ Situation

Other

☒ WNL(within normal limit)

☐ Obsessions

☐ Compulsions

☐ Depersonalization

☐ DeRealism

☐ Denied

Memory

☒ WNL(within normal limit)

☐ Intact

☐ Immediate Impaired

☐ Short Term Impaired

☐ Long Term Impaired

Suicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Homicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Eye Contact

☒ WNL(within normal limit)

☐ Good

☐ Avoidant

☐ Fleeting

☐ Stare

Facial Expression

☒ WNL(within normal limit)

☐ Amiable

☐ Sad

☐ Happy

☐ Angry

☐ Worried

☐ Suspicious

Thought Process

☒ WNL(within normal limit)

☐ Intact

☐ Flight

☐ Blocked

☐ Circumstantial

☐ Loose

☐ Other

☐ Incoherent

☐ Tangential

Delusions

☒ WNL(within normal limit)

☐ Grandiose

☐ Denied

☐ Persecutory

☐ Other

☐ Somatic

☐ Paranoia

☐ Ideas

Demeanor

☒ WNL(within normal limit)

☐ Cooperative

☐ Uncooperative

☐ Guarded

☐ Resistive

☐ Defensive

☐ Friendly

☐ Worried

☐ Boastful

☐ Demanding

☐ Hostile

☐ Irritable

☐ Agitated

☐ Evasive

☐ Dramatic

☐ Seductive

☐ Arrogant

☐ Apathetic

☐ Other

Attention/Concentration

☒ WNL(within normal limit)

☐ Satisfactory

☐ Poor

☐ Easily Distracted

☐ Other

Mental Status Examination

Judgement

☒ WNL(within normal limit) ☐ Good ☐ Limited ☐ Poor

Staff [REDACTED]

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/11/2026 8:13AM

05/11/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	8:00AM	9:00AM
Facility							
R3 Recovery Services							

Treatment Objectives:

- # [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.
[REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.

Assessment Note:

Data: Client presented for session reporting increased stress and anxiety related to a court date scheduled for today. He reported struggling over the weekend after being accused of tampering with his ankle monitor and is currently facing a potential violation of probation. Client expressed frustration and distress, stating he has a UDS result from that date to support that he did not consume substances; however, he reported that his probation officer has declined to review this information. Client identified this situation as a significant contributor to elevated stress levels. He also discussed ongoing psychosocial stressors, including feeling pressure from family to maintain housing in a house rather than an apartment. Client reported that this pressure has led to financially burdensome decisions and ongoing financial strain, despite his stated preference for more affordable housing. Clinician and client processed these stressors and their impact on his emotional functioning. During the final portion of the session, clinician utilized the remaining time to assist client in drafting a letter to his probation officer to support self-advocacy and communication.

Assessment: Client presents with heightened anxiety and stress in response to acute legal stressors and ongoing family and financial pressures. He demonstrates insight into the impact of external expectations on his decision-making and financial situation. Client remains engaged in treatment and shows willingness to utilize problem-solving strategies and self-advocacy skills, as evidenced by active participation in the letter-writing activity. Current stressors increase risk for emotional dysregulation and potential relapse, indicating continued medical necessity for structured therapeutic support and relapse prevention interventions.

Plan: Continue to provide supportive counseling focused on stress management, emotional regulation, and coping with legal and family-related stressors. Treatment will include development of assertive communication and boundary-setting skills related to family expectations, as well as reinforcement of relapse prevention strategies given increased stress. Client will be encouraged to follow through with advocacy efforts, including submission of the letter to his probation officer, and will process outcomes of the court appearance in the next session. Continued focus will be placed on [REDACTED] stability as well as role family plays in the shame he has around his housing.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
☐ Dimension 2: Biomedical Conditions and Complication
☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
☒ Dimension 4: Readiness to Change
☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

☒ WNL(within normal limit) ☐ Clean ☐ Unshaved ☐ Odorous ☐ Disheveled ☐ Other

Mental Status Examination

Affect

☒ WNL(within normal limit)	☐ Appropriate	☐ Inappropriate	☐ Labile	☐ Blunted	☐ Flat
☐ Other	☐ Restricted	☐ Expansive	☐ Euphoric	☐ Anxious	☐ Angry

Hallucinations

☒ WNL(within normal limit)	☐ Visual	☐ Auditory	☐ Tactile	☐ Olfactory	☐ Command
	☐ Denied	☐ Other			

Movement

☒ WNL(within normal limit)	☐ Normal	☐ Tense	☐ Fidgety	☐ Hyperactive	☐ Psychomotor Retardation
☐ Psychomotor Agitation	☐ Pacing	☐ Hand Wringing	☐ Tics	☐ Other	

Mood

☒ WNL(within normal limit)	☐ Anxious	☐ Confused	☐ Depressed	☐ Labile	☐ Fearful
☐ Normal	☐ Euphoric	☐ Angry	☐ Dysthymic	☐ Irritated	☐ Euthymic
	☐ Other				

Speech

☒ WNL(within normal limit)	☐ Normal	☐ Soft	☐ Loud	☐ Pressured	☐ Rapid
	☐ Slowed	☐ Slurred	☐ Incoherent		

Impulse Control

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Insight

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
--	-------------------------------	----------------------------------	-------------------------------

Intelligence

☒ WNL(within normal limit)	☐ Above Average	☐ Average	☐ Below Average
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Oriented

☒ WNL(within normal limit)	☐ Person	☐ Place	☐ Time	☐ Situation
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Other

☒ WNL(within normal limit)	☐ Obsessions	☐ Compulsions	☐ Depersonalization	☐ DeRealism	☐ Denied
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Memory

☒ WNL(within normal limit)	☐ Intact	☐ Immediate Impaired	☐ Short Term Impaired	☐ Long Term Impaired
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Suicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Homicidal Ideation

Mental Status Examination

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Eye Contact

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable ☐ Sad	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
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Thought Process

☒ WNL(within normal limit)	☐ Intact ☐ Flight	☐ Blocked ☐ Circumstantial	☐ Loose ☐ Other	☐ Incoherent	☐ Tangential
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Delusions

☒ WNL(within normal limit)	☐ Grandiose ☐ Denied	☐ Persecutory ☐ Other	☐ Somatic	☐ Paranoia	☐ Ideas
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Demeanor

☒ WNL(within normal limit)	☐ Cooperative ☐ Friendly ☐ Agitated ☐ Other	☐ Uncooperative ☐ Worried ☐ Evasive	☐ Guarded ☐ Boastful ☐ Dramatic	☐ Resistive ☐ Demanding ☐ Seductive	☐ Defensive ☐ Hostile ☐ Arrogant
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Attention/Concentration

☒ WNL(within normal limit)	☐ Satisfactory	☐ Poor	☐ Easily Distracted	☐ Other
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Judgement

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Staff [REDACTED]

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/11/2026 8:11AM

05/18/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	7:30AM	8:30AM
Facility							
R3 Recovery Services							

Treatment Objectives:

[REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Assessment Note:

Data: [REDACTED] attended his individual therapy session and engaged in discussion surrounding his family history and cultural background, including processing his family's immigration to the United States and the trauma they experienced throughout that transition. He reflected on how these experiences have influenced his upbringing and current emotional functioning. [REDACTED] also shared a recent legal stressor, reporting that he received a probation violation which resulted in spending a day in jail. He processed the emotional impact of this event, particularly expressing guilt and concern regarding how his actions have affected his son. [REDACTED] verbalized that he feels he is causing emotional harm to his son due to his continued poor decision-making, indicating increased awareness of consequences. Additionally, he explored trauma related to his relationship with his father, identifying how these past experiences continue to impact his emotional responses and behaviors. Therapist utilized trauma-informed care approaches to support exploration of family history and its connection to current functioning. Cognitive behavioral therapy (CBT) was used to help [REDACTED] identify thought patterns related to guilt and self-perception, while motivational interviewing (MI) interventions were implemented to reinforce accountability and strengthen motivation for change. Supportive therapy and reflective listening were provided to validate emotional experiences and encourage continued openness.

Assessment:

[REDACTED] presents with increased insight into the impact of his past trauma, cultural experiences, and recent legal consequences on his current functioning. He demonstrates growing awareness of how his behaviors affect others, particularly his son, and is beginning to process associated guilt and responsibility. [REDACTED] continues to experience emotional distress related to unresolved trauma and interpersonal relationships, which may contribute to maladaptive coping patterns. His engagement in session and willingness to process difficult topics indicate motivation for change; however, continued support is needed to [REDACTED] strategies, and reduce risk of continued behavioral and legal consequences.

Plan:

Continue individual therapy utilizing CBT, MI, and trauma-informed interventions to support [REDACTED] in processing family-related trauma, [REDACTED] sessions will focus on developing healthier coping mechanisms, improving emotional regulation, and exploring ways to repair relationships [REDACTED] functioning and legal stressors while supporting continued engagement in treatment and recovery-oriented behaviors.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

- ☒ WNL(within normal limit)
- ☐ Clean
- ☐ Unshaved
- ☐ Odorous
- ☐ Disheveled
- ☐ Other

Affect

- ☒ WNL(within normal limit)
- ☐ Appropriate
- ☐ Inappropriate
- ☐ Labile
- ☐ Blunted
- ☐ Flat
- ☐ Restricted
- ☐ Expansive
- ☐ Euphoric
- ☐ Anxious
- ☐ Angry
- ☐ Other

Hallucinations

- ☒ WNL(within normal limit)
- ☐ Visual
- ☐ Auditory
- ☐ Tactile
- ☐ Olfactory
- ☐ Command
- ☐ Denied
- ☐ Other

Movement

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Tense
- ☐ Fidgety
- ☐ Hyperactive
- ☐ Psychomotor Retardation
- ☐ Psychomotor Agitation
- ☐ Pacing
- ☐ Hand Wringing
- ☐ Tics
- ☐ Other

Mood

- ☒ WNL(within normal limit)
- ☐ Anxious
- ☐ Confused
- ☐ Depressed
- ☐ Labile
- ☐ Fearful
- ☐ Euphoric
- ☐ Angry
- ☐ Dysthymic
- ☐ Irritated
- ☐ Euthymic
- ☐ Normal
- ☐ Other

Speech

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Soft
- ☐ Loud
- ☐ Pressured
- ☐ Rapid
- ☐ Slowed
- ☐ Slurred
- ☐ Incoherent

Impulse Control

Mental Status Examination

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Insight

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Intelligence

☒ WNL(within normal limit)	☐ Above Average	☐ Average	☐ Below Average
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Oriented

☒ WNL(within normal limit)	☐ Person	☐ Place	☐ Time	☐ Situation
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Other

☒ WNL(within normal limit)	☐ Obsessions	☐ Compulsions	☐ Depersonalization	☐ DeRealism	☐ Denied
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Memory

☒ WNL(within normal limit)	☐ Intact	☐ Immediate Impaired	☐ Short Term Impaired	☐ Long Term Impaired
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Suicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Homicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Eye Contact

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable ☐ Sad	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
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Thought Process

☒ WNL(within normal limit)	☐ Intact ☐ Flight	☐ Blocked ☐ Circumstantial	☐ Loose ☐ Other	☐ Incoherent	☐ Tangential
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Delusions

☒ WNL(within normal limit)	☐ Grandiose ☐ Denied	☐ Persecutory ☐ Other	☐ Somatic	☐ Paranoia	☐ Ideas
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Demeanor

☒ WNL(within normal limit)	☐ Cooperative ☐ Friendly ☐ Agitated ☐ Other	☐ Uncooperative ☐ Worried ☐ Evasive	☐ Guarded ☐ Boastful ☐ Dramatic	☐ Resistive ☐ Demanding ☐ Seductive	☐ Defensive ☐ Hostile ☐ Arrogant
☐ Irritable					
☐ Apathetic					

Mental Status Examination

Attention/Concentration

- ☒ WNL(within normal limit)
- ☐ Satisfactory
- ☐ Poor
- ☐ Easily Distracted
- ☐ Other

Judgement

- ☒ WNL(within normal limit)
- ☐ Good
- ☐ Limited
- ☐ Poor

Staff

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/19/2026 2:42PM

05/25/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Canceled	7:30AM	8:30AM
Facility R3 Recovery Services							

Treatment Objectives:
holiday-- individuals cancelled.

Assessment Note:
Data: cancel due to holiday

Assessment:

Plan:

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

- ☒ WNL(within normal limit)
- ☐ Clean
- ☐ Unshaved
- ☐ Odorous
- ☐ Disheveled
- ☐ Other

Affect

- ☒ WNL(within normal limit)
- ☐ Appropriate
- ☐ Inappropriate
- ☐ Labile
- ☐ Blunted
- ☐ Flat
- ☐ Restricted
- ☐ Expansive
- ☐ Euphoric
- ☐ Anxious
- ☐ Angry

Mental Status Examination

☐ Other

Hallucinations

☒ WNL(within normal limit)
☐ Visual
☐ Denied
☐ Auditory
☐ Other
☐ Tactile
☐ Olfactory
☐ Command

Movement

☒ WNL(within normal limit)
☐ Psychomotor Agitation
☐ Normal
☐ Pacing
☐ Tense
☐ Hand Wringing
☐ Fidgety
☐ Tics
☐ Hyperactive
☐ Other
☐ Psychomotor Retardation

Mood

☒ WNL(within normal limit)
☐ Normal
☐ Anxious
☐ Euphoric
☐ Other
☐ Confused
☐ Angry
☐ Depressed
☐ Dysthymic
☐ Labile
☐ Irritated
☐ Fearful
☐ Euthymic

Speech

☒ WNL(within normal limit)
☐ Normal
☐ Slowed
☐ Soft
☐ Slurred
☐ Loud
☐ Incoherent
☐ Pressured
☐ Rapid

Impulse Control

☒ WNL(within normal limit)
☐ Good
☐ Limited
☐ Poor

Insight

☒ WNL(within normal limit)
☐ Good
☐ Limited
☐ Poor

Intelligence

☒ WNL(within normal limit)
☐ Above Average
☐ Average
☐ Below Average

Oriented

☒ WNL(within normal limit)
☐ Person
☐ Place
☐ Time
☐ Situation

Other

☒ WNL(within normal limit)
☐ Obsessions
☐ Compulsions
☐ Depersonalization
☐ DeRealism
☐ Denied

Memory

☒ WNL(within normal limit)
☐ Intact
☐ Immediate Impaired
☐ Short Term Impaired
☐ Long Term Impaired

Suicidal Ideation

☒ WNL(within normal limit)
☐ Yes
☐ Plan
☐ Intent
☐ Means
☐ Denied

Homicidal Ideation

☒ WNL(within normal limit)
☐ Yes
☐ Plan
☐ Intent
☐ Means
☐ Denied

Eye Contact

Mental Status Examination

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
	☐ Sad				

Thought Process

☒ WNL(within normal limit)	☐ Intact	☐ Blocked	☐ Loose	☐ Incoherent	☐ Tangential
	☐ Flight	☐ Circumstantial	☐ Other		

Delusions

☒ WNL(within normal limit)	☐ Grandiose	☐ Persecutory	☐ Somatic	☐ Paranoia	☐ Ideas
	☐ Denied	☐ Other			

Demeanor

☒ WNL(within normal limit)	☐ Cooperative	☐ Uncooperative	☐ Guarded	☐ Resistive	☐ Defensive
☐ Irritable	☐ Friendly	☐ Worried	☐ Boastful	☐ Demanding	☐ Hostile
☐ Apathetic	☐ Agitated	☐ Evasive	☐ Dramatic	☐ Seductive	☐ Arrogant
	☐ Other				

Attention/Concentration

☒ WNL(within normal limit)	☐ Satisfactory	☐ Poor	☐ Easily Distracted	☐ Other
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Judgement

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Staff

 MS, LPC

Marleigh Johnson, MS, LPC
05/25/2026 12:29PM

06/01/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	7:30AM	8:30AM
Facility							
R3 Recovery Services							

Treatment Objectives:

[REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Assessment Note:

Data:
Client [REDACTED] attended the individual session and was actively engaged in discussion. Session focused on ongoing family conflict and continued financial stressors. [REDACTED] reflected on the emotional impact of needing to borrow money from his family, identifying feelings of shame, hurt, and frustration related to this dependence. He processed how these financial struggles have strained his relationships and contributed to feelings of inadequacy.

[REDACTED] presents with ongoing emotional distress related to financial instability and strained family relationships. His reflections indicate increased insight into the interconnectedness of his family system and current challenges, though he continues to struggle with ambivalence, resentment, and self-worth concerns. Emotional responses appear appropriate to content and demonstrate growing willingness to process difficult feelings. These stressors present a continued risk for maladaptive coping if not [REDACTED] perceived familial contributions to his situation. However, his engagement and openness in session indicate progress in self-awareness.

Continue to support [REDACTED] in processing family-related conflict and financial stressors while strengthening emotional regulation and coping skills. Focus on developing healthier cognitive frameworks around responsibility, control, and self-worth. Incorporate boundary-setting strategies and communication skills to improve interactions with family members. Encourage exploration of practical problem-solving related to financial stability alongside emotional support. Monitor for changes in mood and coping, and reinforce progress toward increased insight and autonomy.

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

☐ Other

☐ Flat

☐ Angry

 Command

☐ Other

☐ Fearful

☐ Euthymic

 Rapid

 Poor

Insight

Mental Status Examination

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Intelligence

☒ WNL(within normal limit)	☐ Above Average	☐ Average	☐ Below Average
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Oriented

☒ WNL(within normal limit)	☐ Person	☐ Place	☐ Time	☐ Situation
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Other

☒ WNL(within normal limit)	☐ Obsessions	☐ Compulsions	☐ Depersonalization	☐ DeRealism	☐ Denied
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Memory

☒ WNL(within normal limit)	☐ Intact	☐ Immediate Impaired	☐ Short Term Impaired	☐ Long Term Impaired
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Suicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Homicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Eye Contact

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable ☐ Sad	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
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Thought Process

☒ WNL(within normal limit)	☐ Intact ☐ Flight	☐ Blocked ☐ Circumstantial	☐ Loose ☐ Other	☐ Incoherent	☐ Tangential
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Delusions

☒ WNL(within normal limit)	☐ Grandiose ☐ Denied	☐ Persecutory ☐ Other	☐ Somatic	☐ Paranoia	☐ Ideas
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Demeanor

☒ WNL(within normal limit)	☐ Cooperative ☐ Friendly ☐ Agitated ☐ Other	☐ Uncooperative ☐ Worried ☐ Evasive	☐ Guarded ☐ Boastful ☐ Dramatic	☐ Resistive ☐ Demanding ☐ Seductive	☐ Defensive ☐ Hostile ☐ Arrogant
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Attention/Concentration

☒ WNL(within normal limit)	☐ Satisfactory	☐ Poor	☐ Easily Distracted	☐ Other
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Mental Status Examination

Judgement

☒ WNL(within normal limit)

☐ Good

☐ Limited

☐ Poor

Staff [REDACTED]

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/05/2026 1:06PM

06/08/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	7:30AM	8:30AM
Facility	R3 Recovery Services						

Treatment Objectives:

- # [REDACTED] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.
- # [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Assessment Note:

Data:
[REDACTED] attended the individual session and was engaged and reflective throughout. Session focused on his ongoing legal concerns and the emotional impact of preparing for potential incarceration. [REDACTED] shared that he has organized his finances in preparation for the possibility of going to jail, noting that taking these steps has provided him with some sense of relief and increased control. Despite this, he reported persistent fear related to incarceration and its potential impact on his family. [REDACTED] processed feelings of shame connected to his situation and discussed a sense of broken trust within his family system. Therapist utilized supportive reflection, normalization, and cognitive-behavioral interventions to explore these emotional responses, including the interplay between shame, fear, and family dynamics. Insight-oriented discussion was facilitated to help [REDACTED] better understand the emotional weight of these concerns and how they influence his current functioning.

Assessment:
[REDACTED] presents with ongoing anxiety and emotional distress related to legal uncertainty and potential incarceration. While he has taken proactive steps to manage practical concerns, which has improved his sense of control, significant emotional burden remains. Feelings of shame and perceived loss of trust within his family are prominent and appear to impact his self-concept and emotional stability. He demonstrates increasing insight into these dynamics and is able to articulate his internal experiences, though distress remains elevated. Continued support is necessary to process these emotions and reduce their impact on his overall functioning and recovery stability.

Plan:
Continue individual therapy with a focus on processing legal-related anxiety, shame, and family relationship dynamics. Support development of coping skills to manage uncertainty and fear, including grounding and cognitive restructuring techniques. Encourage continued engagement in proactive problem-solving while also [REDACTED] communication within family relationships as appropriate. Monitor mood, anxiety levels, and overall functioning, and reinforce pro

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

☒ WNL(within normal limit)

☐ Clean

☐ Unshaved

☐ Odorous

☐ Disheveled

☐ Other

Mental Status Examination

Affect

☒ WNL(within normal limit)	☐ Appropriate	☐ Inappropriate	☐ Labile	☐ Blunted	☐ Flat
☐ Other	☐ Restricted	☐ Expansive	☐ Euphoric	☐ Anxious	☐ Angry

Hallucinations

☒ WNL(within normal limit)	☐ Visual	☐ Auditory	☐ Tactile	☐ Olfactory	☐ Command
	☐ Denied	☐ Other			

Movement

☒ WNL(within normal limit)	☐ Normal	☐ Tense	☐ Fidgety	☐ Hyperactive	☐ Psychomotor Retardation
☐ Psychomotor Agitation	☐ Pacing	☐ Hand Wringing	☐ Tics	☐ Other	

Mood

☒ WNL(within normal limit)	☐ Anxious	☐ Confused	☐ Depressed	☐ Labile	☐ Fearful
☐ Normal	☐ Euphoric	☐ Angry	☐ Dysthymic	☐ Irritated	☐ Euthymic
	☐ Other				

Speech

☒ WNL(within normal limit)	☐ Normal	☐ Soft	☐ Loud	☐ Pressured	☐ Rapid
	☐ Slowed	☐ Slurred	☐ Incoherent		

Impulse Control

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Insight

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
--	-------------------------------	----------------------------------	-------------------------------

Intelligence

☒ WNL(within normal limit)	☐ Above Average	☐ Average	☐ Below Average
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Oriented

☒ WNL(within normal limit)	☐ Person	☐ Place	☐ Time	☐ Situation
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Other

☒ WNL(within normal limit)	☐ Obsessions	☐ Compulsions	☐ Depersonalization	☐ DeRealism	☐ Denied
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Memory

☒ WNL(within normal limit)	☐ Intact	☐ Immediate Impaired	☐ Short Term Impaired	☐ Long Term Impaired
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Suicidal Ideation

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
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Homicidal Ideation

Mental Status Examination

☒ WNL(within normal limit)	☐ Yes	☐ Plan	☐ Intent	☐ Means	☐ Denied
--	------------------------------	-------------------------------	---------------------------------	--------------------------------	---------------------------------

Eye Contact

☒ WNL(within normal limit)	☐ Good	☐ Avoidant	☐ Fleeting	☐ Stare
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Facial Expression

☒ WNL(within normal limit)	☐ Amiable ☐ Sad	☐ Happy	☐ Angry	☐ Worried	☐ Suspicious
--	--	--------------------------------	--------------------------------	----------------------------------	-------------------------------------

Thought Process

☒ WNL(within normal limit)	☐ Intact ☐ Flight	☐ Blocked ☐ Circumstantial	☐ Loose ☐ Other	☐ Incoherent	☐ Tangential
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Delusions

☒ WNL(within normal limit)	☐ Grandiose ☐ Denied	☐ Persecutory ☐ Other	☐ Somatic	☐ Paranoia	☐ Ideas
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Demeanor

☒ WNL(within normal limit)	☐ Cooperative ☐ Friendly ☐ Agitated ☐ Other	☐ Uncooperative ☐ Worried ☐ Evasive	☐ Guarded ☐ Boastful ☐ Dramatic	☐ Resistive ☐ Demanding ☐ Seductive	☐ Defensive ☐ Hostile ☐ Arrogant
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Attention/Concentration

☒ WNL(within normal limit)	☐ Satisfactory	☐ Poor	☐ Easily Distracted	☐ Other
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Judgement

☒ WNL(within normal limit)	☐ Good	☐ Limited	☐ Poor
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Staff [REDACTED]

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/08/2026 1:45PM

06/15/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Canceled	7:30AM	8:30AM
Facility							
R3 Recovery Services							

Assessment Note:
Data: session was canceled

Assessment:

Plan:

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

- ☒ WNL(within normal limit)
- ☐ Clean
- ☐ Unshaved
- ☐ Odorous
- ☐ Disheveled
- ☐ Other

Affect

- ☒ WNL(within normal limit)
- ☐ Appropriate
- ☐ Inappropriate
- ☐ Labile
- ☐ Blunted
- ☐ Flat
- ☐ Restricted
- ☐ Expansive
- ☐ Euphoric
- ☐ Anxious
- ☐ Angry
- ☐ Other

Hallucinations

- ☒ WNL(within normal limit)
- ☐ Visual
- ☐ Auditory
- ☐ Tactile
- ☐ Olfactory
- ☐ Command
- ☐ Denied
- ☐ Other

Movement

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Tense
- ☐ Fidgety
- ☐ Hyperactive
- ☐ Psychomotor Retardation
- ☐ Psychomotor Agitation
- ☐ Pacing
- ☐ Hand Wringing
- ☐ Tics
- ☐ Other

Mood

- ☒ WNL(within normal limit)
- ☐ Anxious
- ☐ Confused
- ☐ Depressed
- ☐ Labile
- ☐ Fearful
- ☐ Euphoric
- ☐ Angry
- ☐ Dysthymic
- ☐ Irritated
- ☐ Euthymic
- ☐ Normal
- ☐ Other

Speech

- ☒ WNL(within normal limit)
- ☐ Normal
- ☐ Soft
- ☐ Loud
- ☐ Pressured
- ☐ Rapid
- ☐ Slowed
- ☐ Slurred
- ☐ Incoherent

Impulse Control

- ☒ WNL(within normal limit)
- ☐ Good
- ☐ Limited
- ☐ Poor

Insight

- ☒ WNL(within normal limit)
- ☐ Good
- ☐ Limited
- ☐ Poor

Intelligence

- ☒ WNL(within normal limit)
- ☐ Above Average
- ☐ Average
- ☐ Below Average

Mental Status Examination

Oriented

☒ WNL(within normal limit)

☐ Person

☐ Place

☐ Time

☐ Situation

Other

☒ WNL(within normal limit)

☐ Obsessions

☐ Compulsions

☐ Depersonalization

☐ DeRealism

☐ Denied

Memory

☒ WNL(within normal limit)

☐ Intact

☐ Immediate Impaired

☐ Short Term Impaired

☐ Long Term Impaired

Suicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Homicidal Ideation

☒ WNL(within normal limit)

☐ Yes

☐ Plan

☐ Intent

☐ Means

☐ Denied

Eye Contact

☒ WNL(within normal limit)

☐ Good

☐ Avoidant

☐ Fleeting

☐ Stare

Facial Expression

☒ WNL(within normal limit)

☐ Amiable

☐ Happy

☐ Angry

☐ Worried

☐ Suspicious

☐ Sad

Thought Process

☒ WNL(within normal limit)

☐ Intact

☐ Blocked

☐ Loose

☐ Incoherent

☐ Tangential

☐ Flight

☐ Circumstantial

☐ Other

Delusions

☒ WNL(within normal limit)

☐ Grandiose

☐ Persecutory

☐ Somatic

☐ Paranoia

☐ Ideas

☐ Denied

☐ Other

Demeanor

☒ WNL(within normal limit)

☐ Cooperative

☐ Uncooperative

☐ Guarded

☐ Resistive

☐ Defensive

☐ Friendly

☐ Worried

☐ Boastful

☐ Demanding

☐ Hostile

☐ Irritable

☐ Agitated

☐ Evasive

☐ Dramatic

☐ Seductive

☐ Arrogant

☐ Apathetic

☐ Other

Attention/Concentration

☒ WNL(within normal limit)

☐ Satisfactory

☐ Poor

☐ Easily Distracted

☐ Other

Judgement

☒ WNL(within normal limit)

☐ Good

☐ Limited

☐ Poor

Staff

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/16/2026 6:50AM

Date
04/27/2026

Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	5	6:00 PM - 9:00 PM

Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Check-In (MEPS):
Each member took turns sharing their current state using the MEPS framework (Mood, Emotional, Physical, and Spiritual)
Group Discussion: Gratitude in Recovery
After the check-in, the group shifted focus to discussing gratitude and its impact on their recovery journey. Members shared specific things they are grateful for since entering recovery: The group engaged in a reflective discussion about how focusing on gratitude can help shift perspectives and foster a positive mindset. Members noted that recognizing the positives in their lives has been instrumental in maintaining motivation and hope throughout their recovery process. Members were encouraged to keep a gratitude journal to document daily things they are thankful for.
The group agreed to incorporate a gratitude sharing segment at the beginning of future sessions to continue fostering this positive focus. The session concluded with a brief mindfulness exercise centered on gratitude, allowing members to reflect on their individual journeys and the collective support of the group.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: [REDACTED] attended and participated in this being his first group therapy session and began by introducing himself to the rest of the group. member was reluctant or nervous to share much of his history and only provided one word answers which where not clear to this counselor during check in. After check in client became a little more verbal and was able to share his thoughts and feeling related to the assignment on gratitude.

Plan: Continue to provide a safe and supportive environment where member may be able to feel more comfortable in sharing openly and honestly in group therapy sessions as he becomes more connected to the other members.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Reviewer

AKello

Muley JH, MS, LPE

Marleigh Johnson, MS, LPC
04/28/2026 9:49AM

Ashley Kelly, BSW
05/14/2026 2:57PM

Thomas J Close

Tommy Close, MHS
04/28/2026 8:35AM

04/28/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	04/28/2026

Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	4	6:00 PM - 9:00 PM

Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes
Today's session focused on reviewing the various fears experienced in recovery and exploring how these fears impact members' journeys. Participants connected over their shared experiences, discussing the different ways fear manifests and its effects on their recovery process.

The group engaged in a meaningful dialogue about differentiating between fearful beliefs and actionable anxiety. Members identified practical strategies to combat their fears, emphasizing the importance of taking proactive steps to facilitate progress in their recovery.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment: [redacted] expressed significant anxiety related to his fears of financial loss and homele [redacted] His concerns about the future appear to exacerbate his anxiety, indicating a need for strategies to manage these fears and enhance his c

Plan:
Implement grounding techniques and cognitive restructuring to help [redacted] manage anxiety related to uncertainty and financial fears, while also exploring potential resources and support systems; at IOP 3 LOC.

Select the dimension of ASAM [redacted]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [redacted]

Reviewer [redacted]

Jenn Putt

Jenn Putt, MS
05/04/2026 1:56PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/04/2026 7:49PM

04/30/2026

Appt. With	Title	Topic	Date
Tori Barranco, BSW	Group 1 - Thursday PM	Group Therapy	04/30/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	8	6:00 PM - 9:00 PM

Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Today's group focused on Maslow's Hierarchy of Needs. They started the group with prompted check-in questions and then followed that with psychoeducation around the topic and whether members feel they are in survival mode. They then participated in the survival game, which focuses on how our mindsets can change when in survival mode. Members then engaged in a reflective discussion about the activity, discussing how they prioritized who went on the lifeboat, how their personal morals and values influenced their selections, and how their morals and values differ between active addiction and recovery. Finally, members reflected on one way that they would like to work on their morals and values in recovery.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: [REDACTED] appears to be open and willing to engage in the therapeutic process, as evidenced by his participation in the group and his willingness to share his thoughts. His acknowledgment that his basic needs are met indicates a stable foundation, allowing him to focus on psychological growth. The question he raised about self-actualization reflects his curiosity and insight into the complexities of personal development and the impact of life events on mental health. This indicates a level of self-awareness and a desire to understand his own journey.

Plan: Encourage [REDACTED] to continue sharing his thoughts and experiences in group discussions to foster connection and engagement.

Explore the concept of self-actualization further in future sessions, discussing how life events can impact psychological well-being and growth.

Assess [REDACTED]'s specific psychological needs and identify areas for focus in his personal development.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]



Tori Barranco, BSW
05/01/2026 7:45AM



Marleigh Johnson, MS, LPC
05/01/2026 9:00AM

05/04/2026

Appt. With	Title	Topic	Date
Tommy Close, MHS	Group 1 - Monday PM	Group Therapy	05/04/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	8	6:00 PM - 9:00 PM

Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Tonight's group therapy began with each member checking in to where they felt they were at in their recovery and we then focused on the issues surrounding what comes next after a period of sobriety in terms of how to improve our mental health state of mind in order to grow in our self worth and learn ways to be positive productive members of society.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: [REDACTED] attended and participated in this therapy group by sharing that he is about to loose his house to the bank and is concerned not for himself but his wife and two children. He claims that he will have his family go live with other family members and he will just plea to his charges and go to jail for whatever time they intend to give him. He claims that he was arrested for trespassing for using some ones driveway to turn around and then being pulled over and charged with DUI. [REDACTED] claims he has been in treatment before for two other DUI charges, but has never been to jail, but is ready to go if he has to.

Plan: To encourage [REDACTED] to speak to his counselor about the pending foreclosure on his house and also about the charges he has pending so that his counselor can be aware and assist in anyway possible to help him.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff



Tommy Close, MHS
05/05/2026 10:41AM

Reviewer



Marleigh Johnson, MS, LPC
05/06/2026 7:39AM

05/05/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	05/05/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	7	6:00 PM - 9:00 PM

Treatment Objectives

[REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.

Group Notes

In today's session, group members shared their current challenges related to relationships and grief, highlighting the emotional toll these issues have on their recovery journeys. The discussion shifted to the impact of trauma, particularly how adverse childhood experiences (ACEs) shape behaviors and coping mechanisms, with members reflecting on their individual upbringings and the lasting effects on their lives.

Members expressed feelings of frustration and sadness regarding their relationships, emphasizing the importance of accountability and support from others. The group explored how unresolved grief can hinder recovery and the need for open communication in relationships. The conversation delved into the effects of trauma on both mental and physical health, with participants sharing personal experiences that illustrate the connection between childhood trauma and substance use. This understanding fostered a deeper awareness of how past experiences influence current behaviors.

The group discussed the concept of ACEs and how recognizing these experiences can empower members to [redacted] normalization of unhealthy dynamics, which contributed to their struggles with addiction. Members were encouraged to focus on their p[redacted] riences. The discussion highlighted the potential for healing and personal development, emphasizing that recovery is not only about overcoming challenges but about embracing growth and positive change.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment:

[redacted] expressed insight into his difficult childhood, recognizing that he relied on learning as a coping skill to navigate challenging times, yet he also acknowledged his tendency to avoid social interactions and struggle with establishing boundaries, sometimes leading to complete disengagement from others. His reflections indicate a need for further exploration of these avoidance behaviors and their impact on his relationships.

Plan:

Implement strategies to enhance boundary-setting skills and [redacted] sterling a supportive group environment; continue at IOP 3 LOC.

Select the dimension of ASAM [redacted]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [redacted]

Reviewer [redacted]



Jenn Putt, MS
05/12/2026 12:42PM



Marleigh Johnson, MS, LPC
05/12/2026 1:52PM

05/07/2026

Appt. With	Title	Topic	Date
Daniel Joseph, MS, PA-C	Group 1 - Thursday PM	Group Therapy	05/07/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	6	6:00 PM - 9:00 PM

Treatment Objectives

- # [redacted] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.
- # [redacted] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.
- # Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
- # Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Therapist began the group with some light conversation gauging the atmosphere of the group mood before checking in with patients individually. Therapist then focused the majority of their time getting to know the new patients before checking in with the regular patients. After check in was over, therapist had a general discussion with group members revolving around their plans and passions.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: Patient shared that he is court ordered to attend treatment. Patient shared that this is his second time being court ordered to treatment, patient shared that he has learned to accept that he has a problem now as this is the second time it has happened. Patient shared that he justified not being an alcoholic in the past via labs taken by his PCP demonstrating that he has acceptable baseline measurements in regards to liver markers. Patient shared he has struggles with mental health but that he is currently stable. Patient shared that he works in a warehouse and has a one year old and a fourteen year old. Patient shared that he is with his children's mother and that she is supportive. Patient shared financial stress and stress from work and drivers for use.

Plan: Continue to build rapport and encourage patient to explore his acceptance with his alcoholism to ensure that the legal system is not his only motivator. Encourage patient to explore a different work opportunity as this causes him much stress. Encourage patient to build a recovery community. Explore coping mechanisms with patient.

Select the dimension of ASAM [redacted]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff [redacted]

Reviewer [redacted]



Daniel Joseph, MS, PA-C
05/09/2026 5:37PM



Marleigh Johnson, MS, LPC
05/11/2026 8:11AM

05/11/2026

Appt. With	Title	Topic	Date
Tommy Close, MHS	Group 1 - Monday PM	Group Therapy	05/11/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	8	6:00 PM - 9:00 PM

Treatment Objectives

[redacted] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Group Notes

Group members started by completing a MEPS Check-in. A MEPS check-in during group therapy is a structured, holistic opening exercise where participants report how they are doing across four key areas: Mental, Emotional, Physical, and Spiritual. This tool is designed to encourage vulnerability, increase empathy, and build connection among group members by exploring the whole person rather than just their current issues.

Group then participated in an independent writing activity, at which time they answered the following writing prompts:

- The safest space I've ever been in was _____
- What has pain taught you?
- What are five lessons you learned in the last twelve months?
- Is there anything you need to forgive yourself for?
- Are you currently feeling lost or found?

Writing activities are crucial in recovery because they provide a safe, private, and consistent way to process emotions, identify triggers, and reduce stress. By transforming internal chaos into tangible words, writing builds self-awareness, fosters accountability, and helps individuals, such as those in 12-step programs, rewire negative thought patterns into healthy, sober routines. Writing encourages deep self-reflection, helping people in recovery stay grounded in the present moment and focused on their progress. It fosters a better understanding of one's past, present, and future.

-- No patient exercise note is provided --

Assessment:

When answering the question 'are you currently feeling lost or found' they answered "I'm not really sure right now. There's a lot going on in my life. I feel stuck. But I have a great opportunity to make changes."

leaned on the group for additional support this evening, due to having to 'turn himself in tomorrow'. He is hopeful that the judge will 'reinstate bail' but reports that he is 'willing to own up to his mistakes'. His primary struggle at this time is the regret that he carries, due to 'his family feeling pain because of his actions'.

will use his individual sessions to continue to process his responses to his writing prompts, and how his answers impact his recovery.

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Reviewer

A Kelly

Muley, J., MS, LPE

Marleigh Johnson, MS, LPC
05/17/2026 6:24PM

Ashley Kelly, BSW
05/16/2026 2:04PM

05/12/2026

Appt. With	Title
Jenn Putt, MS	Group 1 - Tuesday PM

Topic	Date
Group Therapy	05/12/2026

—

Duration	Status	Group Attendees	Session Times
180 Min	Canceled	6	6:00 PM - 9:00 PM

-- No Treatment Objectives is provided --

Overall, the group discussed themes of procrastination and self-sabotage, exploring how self-sabotage often represents a return to familiar patterns, with the brain perceiving new experiences—even positive ones—as threatening. Members shared personal experiences of how these themes manifested in their lives, and the conversation delved into the connection between procrastination and avoidance, emphasizing the importance of approaching tasks with curiosity, understanding the roots of avoidance, breaking tasks down into smaller, manageable steps, and increasing awareness of self-sabotage patterns.

-- No patient exercise note is provided --

Assessment Note

Probation violation- in jail- expected to be back TH

Staff [Redacted]

Reviewer [Redacted]



Jenn Putt, MS
05/19/2026 12:24PM



Marleigh Johnson, MS, LPC
05/19/2026 2:20PM

05/14/2026

Appt. With
Daniel Joseph, MS, PA-C

Title
Group 1 - Thursday PM

Topic
Group Therapy

Date
05/14/2026

Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	10	6:00 PM - 9:00 PM

Treatment Objectives

- # Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
- # Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes
Therapist checked in with group members to get a gauge on group atmosphere and how their week was going. Therapist began the group with general conversation and open check in which led to a conversation on relationships, boundaries, and what should be looked for in a partner. There were two new group members presents. Therapist spent some time giving them the floor to learn more about their story and have them become more integrated into the group.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
Assessment: Patient sat out the outskirts of the room despite being asked to sit more central. Patient however appeared engaged based on body language and not being on their phone. [Redacted]
group, it should be noted that a barrier could be social and cultural as patient has a thick accent making him difficult to understand.

Plan: Therapist will ask enforce sitting with the group. Therapist will continue to build rapport. Therapist will attempt to get the patient to be more involved. Therapist will help patient brainstorm ways to make his home life more stable as he has mentioned financial, work, and home stresses in the past.

Select the dimension of ASAM [Redacted]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff [Redacted]

Reviewer [Redacted]



Daniel Joseph, MS, PA-C
05/17/2026 2:44PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/17/2026 6:24PM

05/18/2026

Appt. With	Title	Topic	Date
Tommy Close, MHS	Group 1 - Monday PM	Group Therapy	05/18/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	7	6:00 PM - 9:00 PM

Treatment Objectives

[REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.

Group Notes

Group started with a M.E.P.S. check-in. Completing M.E.P.S. (Mental, Emotional, Physical, and Spiritual) exercises during recovery group therapy is crucial because they provide a holistic framework for healing. They help individuals repair the mind-body connection, [REDACTED] p members spent time sharing about their weeks and the intentions they will set for the week to come, to ensure they are on the right

Group members then spent time participating in a healthy competition. Group members were split into two teams for a game of pool. During this game, any group member who was able to score for their team would be asked to answer a question. Once they answered the question, they would choose one additional group member to answer the question. This was an important team building exercise, that allowed group members to learn more about one another.

Group members talked about how participating in sober activities is essential to recovery because it rewires the brain's reward system, prevents relapse by replacing old triggers, and helps build a supportive, like-minded community. They teach you how to manage stress, rediscover joy, and build a meaningful life without substances.

Sober activities provide several core benefits:

- **Natural Dopamine Production:** Addiction alters the brain, making it difficult to feel pleasure without substances. Engaging in hobbies and physical activities triggers natural dopamine release, helping the brain heal.
- **Relapse Prevention:** Boredom and unstructured free time are major relapse triggers. Replacing old habits with constructive activities provides a healthy distraction when cravings hit.
- **Social Connection:** Isolation fuels addiction. Joining sober groups or participating in community events helps you meet people who share your lifestyle and understand your journey.
- **Skill Building:** Recreational activities foster vital life skills such as emotional regulation, goal-setting, and time management, which are critical for navigating everyday stressors.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment:

[REDACTED] shared about the current relationship dynamic between himself and his family. He expressed that he is currently feeling 'overwhelmed'. "My job is at risk since I was in jail for 2 days." [REDACTED] shared how this will impact his family and their current financial struggles.

[REDACTED] shared that this is his first time in treatment. He was not aware that 'this support' existed. He is thankful for his opportunity to make changes in his life.

[REDACTED] answered the following questions during our group activity:

I'm worthy of being loved because..

"I'm doing the best that I can to get my life back to normal. I know that I can do it, but I have to stay focused. I know this is hard for my family."

What change are you most grateful for?

"Everything that I've done for my family. The person I am now is different than who I was. This is the person they deserve."

Plan:

[REDACTED] will continue to participate in sober activities, to reduce the risk of relapse and build healthy social connections. He will use his individual sessions to process ways to continue to enhance his connections in the recovery community.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]

A Kelly

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/25/2026 12:27PM

Ashley Kelly, BSW
05/25/2026 9:19AM

05/19/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	05/19/2026

Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	5	6:00 PM - 9:00 PM

Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Group Notes

During the session, the group engaged in a discussion about stress management and the impact of time pressure on mental well-being, recognizing that our brains often perceive time constraints as a threat, which can exacerbate feelings of stress. Participants shared their experiences with managing stress related to work and personal relationships, highlighting the importance of setting boundaries and prioritizing self-care.

We explored the concept of the time matrix, emphasizing the distinction between urgent and tasks, and discussed strategies for better managing time and energy. Group members reflected on their tendencies to get caught up in less productive activities while expressing a desire to allocate more time for planning and long-term visioning. Various resources were suggested, including libraries, coffee shops, parks, and coworking spaces, as alternative environments that can foster focus and limit distractions. Overall, the session provided valuable insights into how changing settings and implementing time management techniques can help alleviate stress and enhance productivity.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment
[REDACTED] exhibited moderate emotional engagement during the session as he explored his work-related stress and anxiety stemming from the uncertainty of his legal situation, noting that he often gets caught up in "what if" scenarios. He also acknowledged a tendency to avoid discussing his feelings, opting instead to cope by going to a bar and drinking in the past, indicating a need for healthier coping strategies and improved communication skills.

Plan
Encourage [REDACTED] to practice sharing his feelings in a supportive environment while implementing coping strategies to replace past behaviors with healthier alternatives; continue at IOP 3 LOC.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications

- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Reviewer



Jenn Putt, MS
05/22/2026 7:59AM



Marleigh Johnson, MS, LPC
05/22/2026 11:52AM

05/21/2026

Appt. With	Title	Topic	Date
Daniel Joseph, MS, PA-C	Group 1 - Thursday PM	Group Therapy	05/21/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	12	6:00 PM - 9:00 PM

Treatment Objectives

- # [REDACTED] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.
- # [REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.

Group Notes

Therapist started the group with a general conversation talking with the participants to gauge group atmosphere. Therapist reinforced facility and group norms, patients complied and adhered throughout the group. Therapist then opened the floor for new members to introduce themselves and tell some of their story. Therapist then led a group on SMART (Specific, Manageable, Attainable, Realistic, Timely) goals, patients stated their goals. Therapist gave feedback on the goals that were given and will use this as a touch point in future groups.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: Patient shared that he continues to work on his financial stresses. Patient remains at a job he is unhappy with. Patient works very hard for his family but could not articulate doing anything personally for himself.

Plan: Continue to encourage the patient to seek help with his financial struggles, to find a new job, and to specifically seek some personal time to take care of himself. Continue to build rapport.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff

Reviewer



Daniel Joseph, MS, PA-C
05/26/2026 9:06PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
05/28/2026 7:44AM

05/25/2026

Appt. With	Title	Topic	Date
Tori Barranco, BSW	Group 1 - Monday PM	Group Therapy	05/25/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	6	6:00 PM - 9:00 PM

Treatment Objectives

[REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Group Notes

Today's group session focused on the role of personal values in supporting motivation and engagement in recovery. Group members participated in an initial check-in, discussing their weekend experiences and processing emotions related to the Memorial Day holiday. Members then completed a values-based reflective worksheet to identify core values and explore how these values are demonstrated in their daily lives and recovery process. The session also included a therapeutic tie-dye activity in which participants associated specific colors with identified personal values as a means of creative expression and reinforcement of recovery-oriented themes. The group concluded with a debriefing discussion in which members reflected on insights gained and shared their experiences of the activity.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment:

[REDACTED] shared that his battery is currently at 60%. "I'm staying hopeful and remaining open to whatever is to come in life. I don't have control over a lot and I'm focused on what I can control."

What are the most important values you want to live by in your recovery right now?

"Positivity, focus, and acceptance are all important to me. I need to stay focused and accept the results and consequences of my actions. I feel like I'm finally doing that."

When have you recently acted in a way that matched your values? What happened?

"My values are to stay positive. I'm trying on a daily basis. I'm in a high risk of losing my job but I'm still positive that it is happening for the better. I have to see the positive in things."

What situations make it hardest for you to stay aligned with your values?

"The stress in my life. Having to be responsible for my family and what happens to them."

What is one small change you could make this week to better live according to your values?

"I will be working on my financial management this week. I need to make sure that my family is well and do my best to save our home."

Plan:

[REDACTED] will continue to process his values in his individual sessions to identify areas for growth, to ensure that he is aligned with his values during his recovery journey.

Select the dimension of ASAM

[REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Reviewer



Ashley Kelly, BSW
05/28/2026 12:41PM



Marleigh Johnson, MS, LPC
05/26/2026 1:47PM



Tori Barranco, BSW
05/26/2026 11:56AM

05/26/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	05/26/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	8	6:00 PM - 9:00 PM

Treatment Objectives

[REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.

Group Notes

During the session, the group engaged in a meaningful discussion about hindsight bias, reflecting on how past experiences and decisions can influence current perceptions and judgments. Members were encouraged to practice self-compassion and grace, particularly when acknowledging that their judgments may have been impaired due to stress or substance use, reinforcing the idea that recovery is a journey filled with learning opportunities.

Participants shared personal experiences, such as navigating relationships and boundaries while balancing family life and recovery, and the importance of recognizing triggers in their environments. The group highlighted moments of strength, like proactively seeking support during challenging situations and offering comfort to peers in distress, which fostered a sense of community and mutual encouragement. Overall, the conversation emphasized the significance of understanding one's emotional responses, forgiving oneself for past mistakes, and celebrating progress in the recovery journey

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment:
[REDACTED] expressed regrets regarding his legal situation and reflected on a friendship that has led to feelings of betrayal, indicating a deeper exploration of trust and its impact on his relationships. He also examined his feelings about the accident, recognizing how impaired decision-making under the influence has contributed to his current circumstances, highlighting the need for greater awareness and accountability in his choices.

Plan:
Facilitate discussions around trust and decision-making, integrating strategies to enhance self-awareness and accountability, while exploring the emotional consequences of his past actions; consider utilizing role-playing scenarios to practice making healthier choices in challenging situations. Continue at IOP 3 LOC.

Select the dimension of ASAM [REDACTED]

☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential

☐ Dimension 2: Biomedical Conditions and Complication

- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Reviewer



Jenn Putt, MS
05/29/2026 2:49PM



Marleigh Johnson, MS, LPC
06/01/2026 7:55AM

05/28/2026

Appt. With	Title	Topic	Date
Daniel Joseph, MS, PA-C	Group 1 - Thursday PM	Group Therapy	05/28/2026

Add-On Service

--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	7	6:00 PM - 9:00 PM

Treatment Objectives

- # Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
- # Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Therapist tends to run groups of a more serious tone. Patients have participated well during these groups and therefore has rewarded them with a movie group. Before watching the movie with the patients, therapist asked a new individual to introduce themselves. After the new patient introduced themselves and conveyed some of their story, therapist opened the floor for patient share any pressing matters they may be experiencing. Patients declined to share at this time. Therapist then allowed the patients to vote on what recovery related movie they wanted to watch, patients chose to watch *Beautiful Boy*. Therapist will spend the next week exploring the patients feelings in regard to the movie and relevant themes displayed.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: Patient presented to group and was semi compliant with the movie group evident by appearing to fall asleep at times during group.

Plan: Build rapport with patient, encourage patient to engage with the group, access the knowledge gained from the movie watched during group today.

Select the dimension of ASAM

- ☒ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☒ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff

Reviewer



Daniel Joseph, MS, PA-C
06/02/2026 9:14PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/03/2026 5:51AM

06/01/2026

Appt. With	Title	Topic	Date
Tommy Close, MHS	Group 1 - Monday PM	Group Therapy	06/01/2026

Add-On Service

--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	10	6:00 PM - 9:00 PM

Treatment Objectives

- # [REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.
- # [REDACTED] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.
- # [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.
- # [REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.
- # Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
- # Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes

Check-In: Each member was invited to share their current feelings regarding their recovery journey. The check-in allowed participants to express their thoughts and emotions, fostering a supportive environment. Group Discussion: The primary focus of the group discussion centered around the purpose of group therapy. Participants were reminded that group therapy is a space for healing, learning, and mutual support. Each member has a responsibility to contribute to the group by sharing openly and honestly while being considerate of others. Key Points Discussed: The importance of vulnerability in sharing experiences to foster deeper connections. How honesty can lead to breakthroughs in understanding oneself and others. Group Rules: To maintain a respectful and productive atmosphere, we reviewed and reinforced the following group rules: No Cross Talking: Members are encouraged to listen to one another without interrupting or engaging in side conversations. No [REDACTED]. Conclusion: The group concluded with a reminder of the importance of these rules in creating a safe space for everyone. Participants were encouraged to reflect on their experiences and consider how they can contribute positively to the group dynamic moving forward.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment: [REDACTED] attended and participated by sharing with the group that he had a court hearing that did not go too well and it was just negotiations on a plea deal that did not go in his favor. Therefore he claims they will go back to court in two month's and re-negotiate. [REDACTED] went on to share that he had a meeting with his family and one of his sister's had offered to help him and his family out with a loan for their financial needs.

Plan: To continue encouraging [REDACTED] to attend all scheduled group and individual therapy sessions as well as attending twelve step meetings when he can.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]

Thomas J Close

Tommy Close, MHS
06/02/2026 1:25PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/03/2026 5:51AM

06/02/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	06/02/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	10	6:00 PM - 9:00 PM

Treatment Objectives

[REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.

Group Notes
In today's session, the group engaged in a discussion about self-esteem and self-worth, focusing on the differences between internal and external validation and the challenges associated with seeking external approval. Members shared their experiences with self-worth, reflecting on how it has influenced their behaviors and led them to pursue achievements as a means of bolstering low self-esteem, while also examining the impact of the inner critic on their recovery and exploring strategies to cultivate self-compassion.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment:
[REDACTED] expresses difficulty in asserting himself at work, feeling unable to say no or ask for help due to a perceived obligation to his leadership, which contributes to his anxiety about making mistakes in his recovery. His decision to stay home to avoid drinking risks highlights his awareness of potential triggers, but it also raises concerns about isolation and frustration, which could jeopardize his long-term recovery.

Plan:
Encourage [REDACTED] to develop assertiveness skills to communicate his needs at work and explore healthy coping strategies for managing anxiety, while also identifying social activities that provide meaning and connection outside of his home environment. Continue at IOP 3 LOC.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]

Jenn Putt

Jenn Putt, MS
06/08/2026 1:49PM

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
06/09/2026 7:10AM

06/04/2026

Appt. With Daniel Joseph, MS, PA-C	Title Group 1 - Thursday PM	Topic Group Therapy	Date 06/04/2026
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Add-On Service
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Duration 180 Min	Status Attended	Group Attendees 15	Session Times 6:00 PM - 9:00 PM
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Treatment Objectives

Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes
Therapist started the group with general conversation to gauge group atmosphere. Therapist then opened the floor to the new patients allowing them to introduce themselves and tell some of their story. Therapist then proceeded to continue their group from last week and discussed the movie that was viewed with the patients.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
Assessment:

Patient was present and actively engaged in today's group session. He participated in group discussion and demonstrated appropriate peer interaction throughout the meeting. Patient discussed the movie from last week sharing that he felt the main character lacked community support, effort, and that his family was too enabling.

Plan:

Patient is starting to share more without direct prompt. I want to continue to encourage this so the patient can build a community within our system. Continue to check in with the patients goals in relation to life stability.

Select the dimension of ASAM [REDACTED]

- ☒ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☒ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff [REDACTED]



Daniel Joseph, MS, PA-C
06/06/2026 2:13PM

Reviewer [REDACTED]



Marleigh Johnson, MS, LPC
06/08/2026 9:17AM

06/08/2026

Appt. With Tommy Close, MHS	Title Group 1 - Monday PM	Topic Group Therapy	Date 06/08/2026
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Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	11	6:00 PM - 9:00 PM

Treatment Objectives
-- No Treatment Objectives is provided --

Group Notes
-- No group note is provided --

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
-- No assessment note is provided. --



06/09/2026

Appt. With	Title	Topic	Date
Jenn Putt, MS	Group 1 - Tuesday PM	Group Therapy	06/09/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	9	6:00 PM - 9:00 PM

Treatment Objectives
[REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Group Notes
Today's group focused on the topic of triggers in recovery, where we explored the definition of triggers and identified different types that can impact members' journeys. Participants shared their personal triggers and engaged in discussions about the importance of raising awareness of these triggers and creating proactive plans to [REDACTED]

Members provided support to one another by discussing specific triggers and sharing strategies for coping. Each participant was encouraged to define actionable steps they could take when encountering their triggers, fostering a collaborative environment aimed at enhancing individual recovery strategies.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment:
[REDACTED] reports experiencing increased stress from work due to changes in team composition, which have impacted his workload and contributed to feelings of overwhelm. He actively sought and received support from the group regarding strategies to manage this stress, including the importance of not overextending himself and reframing his responsibilities as management's decisions rather than personal failings.

Plan:
Encourage [REDACTED] to implement the strategies discussed in the group to manage his workload effectively while reinforcing the importance of setting boundaries and maintaining a healthy work-life balance. Continue at IOP 3 LOC.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]



Jenn Putt, MS
06/11/2026 1:36PM



Marleigh Johnson, MS, LPC
06/15/2026 1:45PM

06/11/2026

Appt. With	Title	Topic	Date
Tori Barranco, BSW	Group - Thursday PM	Group Therapy	06/11/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	9	6:00 PM - 9:00 PM

Treatment Objectives

- # [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.
- # Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.
- # Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Group Notes
The group focused on relapse prevention and increasing awareness of the relapse process. Group members received education regarding the three stages of relapse: emotional relapse, mental relapse, and physical relapse. Participants engaged in a facilitated discussion exploring warning signs, triggers, thoughts, emotions, and behaviors associated with each stage of relapse. Group members actively shared personal experiences and provided peer feedback, fostering insight into individual relapse patterns and prevention strategies. Participants also completed a relapse prevention worksheet designed to identify personal risk factors, coping skills, and protective factors. The session concluded with members processing their responses and discussing key insights gained from the activity.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment: [REDACTED] participated in the group but seemed preoccupied with his thoughts. He shared that his emotional relapse is characterized by reduced appetite and excessive sleeping. He mentioned that he often rationalizes his desire to drink by telling himself that he will only do it once or by making excuses related to family stress. He acknowledged that once he enters a mental relapse, it quickly leads to a physical relapse. Managing a good routine with exercise and healthy eating is important to him .

Plan: Create a structured daily schedule that includes regular meals, physical activity, and time for self-care to combat oversleeping and poor eating habits.

- Select the dimension of ASAM [REDACTED]
- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
 - ☐ Dimension 2: Biomedical Conditions and Complication
 - ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
 - ☒ Dimension 4: Readiness to Change
 - ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
 - ☒ Dimension 6: Recovering/Living Environment

Staff [REDACTED]

Reviewer [REDACTED]



Tori Barranco, BSW
06/12/2026 1:02PM



Marleigh Johnson, MS, LPC
06/15/2026 1:44PM

06/15/2026

Appt. With	Title	Topic	Date
Tommy Close, MHS	Group 1 - Monday PM	Group Therapy	06/15/2026

Add-On Service

--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	5	6:00 PM - 9:00 PM

Treatment Objectives

- # [REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.
- # [REDACTED] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.
- # [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.
- # [REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.

Group Notes

Tonight's group therapy began with an Alumni member coming into group to share their recovery with the other members of the group, sharing their history of addiction, prior treatment attempts and developments in recovery since completing treatment. The group members then participated in each member sharing the things they are grateful for today in recovery.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

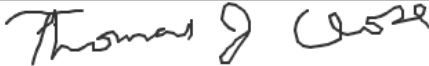
Assessment: [REDACTED] attended and participated in this therapy group session by listening to the Alumni speaker and sharing about the things he is most grateful for. [REDACTED] shared with the other members that he never believed he had a problem with addiction until coming to R3 recovery services. He claims to be grateful for his family and having the ability to remain sober with the help of his counselor and the other members of the community. He shared that having the ability to continue affording to provide for himself and his family is something he had worried about with all his legal problems but he is finding a way. [REDACTED] shared that he is grateful and appreciates his own honesty when it comes to sharing in group and has gratitude towards all the members of the community who offer feedback and suggestions towards his recovery.

Plan: To continue to encourage [REDACTED] to continue sharing in all group and individual therapy sessions, while accepting feedback and information provided by the older members of the group and his counselor.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff [REDACTED]



Tommy Close, MHS
06/16/2026 7:55AM

TRANSPORT APPOINTMENT

-- No Transport Appointments --

EMERGENCY CONTACTS

Name	Email				End Date
Daren	dsharp@lanastercountypacourt.gov	(717) 925-3939	Yes	04/23/2026	04/22/2027
Radhika Cuwan		(717) 615-5313	Yes	04/23/2026	04/22/2027

INTAKE: ADMISSIONS

Intake Time

Start Time:04/23/2026 @ 03:00 PM

End Time:04/23/2026 @ 05:00 PM

Reason for Seeking Treatment

Reason for Seeking Treatment

Reason for seeking treatment?
I got a 3rd DUI and they are court mandating me to get an evaluation

What is your primary drug of choice?
Alcohol

Describe current cravings and how they are managed:
I am on Vivitrol and that is helping with my cravings.

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Alcohol	0	Drinking	Unknown I would binge drink.	02/14/2026	20	9	buy it.	Binge

Previous Treatments

TX Center Name	Duration	Level Of Care	What was the response to previous treatment? How long did you stay sober after discharge?
Giesinger Hospital	3 days	3.5	Detox. feb 15 2026.
Reading Hospital	7 days	3.5	I did detox because I drank too much. 2015

Consequences of Use - BPSA

Have you ever had a blackout?
☐ Yes ☒ No

Have you ever tried to hide your substance use from others?
☐ Yes ☒ No

Have you ever tried to cut down or stop your substance use?

☒ Yes ☐ No

Have you ever used substances in the morning?

☐ Yes ☒ No

Do you feel you have a problem with substances?

☒ Yes ☐ No

Have you ever driven while under the influence of substances?

☒ Yes ☐ No

Have you ever lost friends or relationships because of substance use?

☐ Yes ☒ No

Has anyone ever said they were concerned about your substance use?

☒ Yes ☐ No

Have you decreased your social/recreation activities because of substances?

☐ Yes ☒ No

Have you ever felt bad or guilty about your substance use?

☒ Yes ☐ No

What negative consequences have you experienced due to drug or alcohol abuse?

"Little trouble"

Legal troubles

Priority Population Screening Questions*

Priority Population Screening Questions

(The following questions help us determine urgency of admission and ensure timely access to care)

Pregnancy & Substance Use

Are you currently pregnant?

☐ Yes ☒ No ☐ Unsure

If Yes, are you currently using any substances (including alcohol or drugs)?

☐ Yes ☒ No

If Yes, have you injected any substances within the past year?

☐ Yes ☒ No

Injection Drug Use

Have you injected any drugs within the past year?

☐ Yes ☒ No

Overdose History

Have you experienced an overdose within the past 12 months?

☐ Yes ☒ No

If Yes, approximately when did the overdose occur?

Veteran Status

Have you ever served in the United States Armed Forces?

☐ Yes ☒ No

Notes/Comments

TB Risk Screening Questions*

TB Risk Screening Questions

Have you traveled extensively (more than 4 weeks) outside the U.S. in the last five years to high TB-incidence areas (Asia, Africa, South America, Central America)?

☐ Yes ☒ No

Are you an immigrant from a high TB-risk foreign country (includes countries in Asia, Africa, South America, and Central America)?

☐ Yes ☒ No

Have you resided in any of these facilities in the past year: jails, prisons, shelters, nursing homes and other long-term care facilities such as rehabilitation centers?

☐ Yes ☒ No

Have you had any close contact with someone diagnosed with TB?

☐ Yes ☒ No

Have you been homeless within the past year?

☐ Yes ☒ No

Have you ever injected drugs?

☐ Yes ☒ No

Do you or anyone in your household currently have the following symptoms, such as a sustained cough for two or more weeks, coughing up blood, fever/chills, loss of appetite, unexplained weight loss, fatigue, night sweats?

☐ Yes ☒ No

Do you currently have or anticipate having any condition that would decrease your immune system? (Examples: HIV infection, organ transplant recipient, treatment with TNF-alpha antagonist (e.g. infliximab, etanercept, others), steroids (equivalent dose of Prednisone 15mg/day for one month or longer) or any other immunosuppressive medications)

☐ Yes ☒ No

Would you like a referral to a TB clinic for support or testing?

☐ Yes ☒ No

Notes/Comments

Contagious Disease Screening*

Contagious Disease Screening

Hepatitis Status

Have you ever been tested for Hepatitis B?

☐ Yes ☒ No ☐ Unsure

If Yes, what were the results?

☐ Positive ☐ Negative ☒ Unsure

Have you ever been tested for Hepatitis C?

☐ Yes ☒ No ☐ Unsure

If Yes, what were the results?

☐ Positive ☐ Negative ☒ Unsure

Injection Drug Use

Have you ever injected drugs (used a needle to inject any substance), even one time?

☐ Yes, currently ☐ Yes, in the past ☒ No

Intranasal Drug Use

Have you ever used drugs intranasally (snorted substances such as cocaine, heroin, fentanyl, etc.)?

☐ Yes, currently ☐ Yes, in the past ☒ No

HIV Status

Have you ever been diagnosed with HIV?

☐ Yes ☒ No ☐ Prefer not to answer

If Yes, are you currently receiving medical care for HIV?

☐ Yes ☒ No

Have you had sexual contact with:

- ☐ A person living with HIV
- ☐ A person whose HIV status you did not know
- ☐ A person who has multiple sexual partners
- ☒ None of the above

Do you currently have multiple sexual partners?

- ☐ Yes
- ☒ No

Have you ever shared needles, syringes, or other drug preparation equipment with another person?

- ☐ Yes
- ☒ No

Have you ever been diagnosed with viral hepatitis (Hepatitis A, B, or C) or a sexually transmitted infection?

- ☐ Yes
- ☒ No
- ☐ Unsure

Incarceration History

Have you ever been incarcerated (jail, prison, detention facility)?

- ☐ Yes
- ☒ No

Referral and Education

Would you like assistance with any of the following services?

- ☐ Referral for Viral Hepatitis testing
- ☐ Referral for HIV testing
- ☐ Education about Hepatitis or HIV prevention and treatment
- ☐ Support services related to Hepatitis or HIV care
- ☒ Not at this time

Notes/Comments

Suicide/Homicidal Risk Screen - BPSA

Have you ever experienced HI or SI or Self Harm? If yes, please explain. Include plans, dates, methods, lethality.
I had been 302ed in February because I said something to the officer when I was drunk, but I never really intended to hurt myself.

Do you have any current HI or SI or Self Harm? If yes, please explain.
None right now.

Digitally Signed by



Caleb Madison, BS 04/23/2026 2:24PM

Reviewer



Marleigh Johnson, MS, LPC 04/27/2026 6:59AM

----- END OF ADMISSIONS INTAKE -----

INTAKE: FACE TO FACE**Other Information**

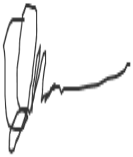
First Contact Date:	<u>04/13/2026</u>	Prospective Client Intake By:	<u>Ethan Jandrew, BA</u>
Referral Company :	<u></u>	Referral Contact :	<u></u>
Admission Date :	<u>04/23/2026</u>	Discharge/Transfer:	<u></u>
Facility Name :	<u>R3 Recovery Services</u>	Prospective Client Status :	<u>Active</u>
Primary Clinician :	<u>Marleigh Johnson</u>	Prospect Status :	<u>Anticipated Admission</u>
Level of care :	<u>GOP</u>	Treatment :	<u></u>

ROI Contacts

Name	Relation	City	State	Country	Mobile	ROI	Emergency Contact
Radhika Cuwan	Wife	Litiz	Pennsylvania	United States	(717) 615-5313	Yes	Yes
Daren	Other				(717) 925-3939	Yes	No

Insurance

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care	 Singh Chuwan	Self	None Specified	771216145812	Primary		Active

Digitally Signed by

Caleb Madison, BS 04/23/2026 2:28PM

Reviewer 

Marleigh Johnson, MS, LPC 04/27/2026 6:59AM

----- END OF FACE TO FACE INTAKE -----

INTAKE: BIO PSYCHO SOCIAL

Reason for Seeking Treatment

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Reason for seeking treatment?

I got a 3rd DUI and they are court mandating me to get an evaluation

What is your primary drug of choice?

Alcohol

Describe current cravings and how they are managed:

I am on Vivitrol and that is helping with my cravings.

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Alcohol	0	Drinking	Unknown I would binge drink.	02/14/2026	20	9	buy it.	Binge

Treatment and Recovery History - BPSA

Have you ever been treated for substance abuse?

☐ Yes ☒ No

Facility	Name of Provider	ASAM LOC	Approximate Date	Duration	Outcome

Have you ever attended a meeting for AA or NA?

☒ Yes

☐ No

Have you ever had a sponsor?

☐ Yes

☒ No

Longest length of sobriety?

18th months after my second DUI

How did you remain sober?

I had an interlock and I needed to use it to start the car.

Why did you relapse?

I just started trying to drink once in a while. I made the choice to only drink at my house.

Other addictive behaviors:

None

Describe your strengths that will help you with recovery:

I want to get this taken care of. It is causing a lot of issues in my life and I don't want to have these issues again.

Describe your challenges or barriers to recovery:

Sometimes life has ups and downs. I have frustrations. When those come I want to drink.

Gambling

Gambling

Do you gamble? If yes, what type and how often?

None

Do you feel unable to control how often you gamble?

None

Do you feel unable to control how much money you bet?

None

Do you lie or minimize your gambling to others?

None

Has your gambling caused any financial, relationship, or work problems?

None

Consequences of Use - BPSA

Have you ever had a blackout?

☐ Yes

☒ No

Have you ever tried to hide your substance use from others?

☐ Yes

☒ No

Have you ever tried to cut down or stop your substance use?

☒ Yes

☐ No

Have you ever used substances in the morning?

☐ Yes

☒ No

Do you feel you have a problem with substances?

☒ Yes

☐ No

Have you ever driven while under the influence of substances?

☒ Yes

☐ No

Have you ever lost friends or relationships because of substance use?

☐ Yes

☒ No

Has anyone ever said they were concerned about your substance use?

☒ Yes

☐ No

Have you decreased your social/recreation activities because of substances? ☐ Yes ☒ No

Have you ever felt bad or guilty about your substance use? ☒ Yes ☐ No

What negative consequences have you experienced due to drug or alcohol abuse?
Legal troubles

Family Information - BPSA

Are you Married?

- ☐ Divorced
- ☐ Separated
- ☐ Widowed
- ☐ Never married
- ☐ Living with partner
- ☐ In relationship
- ☒ Married

Number of Marriages: 2008

Does your spouse use substances? ☐ Yes ☒ No ☐ NA

Do you have children?

Name	Age	Sex	Quality of Relationship	In your custody?
Radhav	14	male	it's good. When I am drunk he won't talk to me.	Yes
Gawrave	10 months	male	Good	yes

Name	Age	Sex	Quality of Relationship	In your custody?

☐ Check here if no children

How has your relationship with family/significant other been affected by your substance use?
She's not mean to me about my drinking, but she wants me to stop drinking. I was told that I am infertal but my wife got pregant and I started to sprial because I thought it might not be mine. Every time she got pregant after that I relapsed and started to drink heavily every night. She told me I could get a DNA test but I said not because now that he's 14 I don't want to know.

Would you like to have your family involved in your treatment?
None

Describe your current living environment and how it affects your recovery:
No alcohol at home. I am not allowed to drink. My wife is supportive.

Family History - BPSA

Has anyone in your family ever been diagnosed with a mental health condition?
☐ Yes ☒ No

*If yes, fill in the blanks below

Family Member	Diagnosis	Type of Treatment

Does anyone in your family have a history of substance/alcohol abuse?
☐ Yes ☒ No

*If yes, fill in blanks below

Family Member	Type of Drug	Outcome

Does anyone in your family have a serious illness or chronic medical condition?

☐ Yes ☒ No

*If yes, fill in blanks below

Family Member	Diagnosis

Please describe your home environment as a child

It was tough. My dad left when I was 7. My mom rasied me herself. She wokred a lot. I grew up in napal

Please describe yourself as a child

I was good at school. I was will behaved. I never did any sports. I was lucky that I got a chance to go to a school. I immigrated in 2010.

Describe your current relationship with your parents

Mom - good.

Dad - I don't know where he is. I have no relationship with her.

Do you have siblings? ☒ Yes ☐ No

Brothers

How many and what ages?

Mon - 52
Sita - 48
Surya - 42

Sisters

How many and what ages?

Kharka - 50
Hari - 38
B [REDACTED]

Describe your relationship with your siblings

We are all close. I am cool with everyone but some of them aren't cool with me because they think I am a bad drunk.

Who is supportive of your recovery

My wife.

Mental Health History/Current Symptoms - BPSA

Do you have a history of any mental health diagnoses or symptoms?

Depression

Have you ever received
treatment/counseling for mental health
issues?

☐ Yes ☒ No

Facility Name	Location	Therapist	Diagnosis/Medication	Reason for Treatment	Response to Treatment/ Intervention	Date

Facility Name	Location	Therapist	Diagnosis/Medication	Reason for Treatment	Response to Treatment/ Intervention	Date

Are you currently experiencing any mental health symptoms?

☐ Yes ☒ No

Please elaborate on current symptoms and how your daily functioning is impacted:

No, I feel fine. I take prozac

Do you use drugs or alcohol to relieve mental health symptoms?

☒ Yes ☒ No ☐ NA

Are you currently seeing a psychiatrist?

☐ Yes ☒ No

If so, what is their name and when is your next scheduled appointment?

Suicide/Homicidal Risk Screen - BPSA

Have you ever experienced HI or SI or Self Harm? If yes, please explain. Include plans, dates, methods, lethality.

I had been 302ed in February because I said something to the officer when I was drunk, but I never really intended to hurt myself.

Do you have any current HI or SI or Self Harm? If yes, please explain.

None right now.

C-SSRS Screener (Safety Plan for mod/high risk) Outpatient

COLUMBIA-SUICIDE SEVERITY RATING SCALE
Screen Version - Recent

Suicide Ideation Definitions and Prompts

Ask Questions 1 and 2

Month

1) Wish to be dead

Have you wished you were dead or wished you could go to sleep and not wake up?

☐ Yes ☒ No

2) Current suicidal thoughts

Have you actually had any thoughts of killing yourself?

☐ Yes
☒ No

If YES to 2, ask questions 3, 4, 5, and 6. If NO to 2, go directly to question 6.

Month

3) Suicidal thoughts w/ Method (w/no specific Plan or Intent or act)

Have you been thinking about how you might do this?

☐ Yes
☒ No

4) Suicidal Intent without Specific Plan

Have you had these thoughts and had some intention of acting on them?

☐ Yes
☒ No

5) Intent with Plan

Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?

☐ Yes
☒ No

6) C-SSRS Suicidal Behavior: "Have you ever done anything, started to do anything, or prepared to do anything to end your life?"

Examples: Collected pills, obtained a gun, gave away valuables, wrote a will or suicide note, took out pills but didn't swallow any, held a gun but changed your mind or it was grabbed from your hand, went to the roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Lifetime

☐ Yes
☒ No

If "YES" Was it within the past 3 months?

Past 3 Months

☐ Yes
☒ No

C-SSRs Score:

☐ High ☐ Moderate ☐ Low ☒ No Identified Risk

Comments:

Columbia-Suicide Severity Rating Scale

Intensive Outpatient Programming/ Outpatient Programming: Interventions for each risk level may include the following. Risk level and interventions must always be documented in the electronic health record.

High	☐ Referral to local emergency room ☐ Referral to crisis team ☐ Referral and evaluation by a mental health professional or psychiatry (internal or external) ☐ Re-evaluate for level of care; reassess at treatment plan review for continuing in program ☐ Engagement with family/significant other ☐ Personal safety plan ☐ Create/update IAPP ☐ Give crisis team/suicide risk number at discharge
Moderate	☐ Referral and evaluation by a mental health professional or psychiatry (internal or external) ☐ Engagement with family/significant other ☐ Personal safety plan ☐ Create/update IAPP ☐ Give crisis team/suicide risk number at discharge ☐ Reassess at treatment plan review
Low	☐ Mental health resources and support provided - internal or external ☐ Give crisis team/suicide risk number at discharge ☐ Provide information about suicide risk warning signs ☒ Re-assess at treatment plan review

Trauma Screening - BPSA

Were you the victim of any Trauma, Abuse, Neglect or Exploitation? If yes, please provide details
None

Have there been any traumatic events(that you have witnessed or experienced) that you feel have affected you over the course of your life?
I lived in a country that had a ten year civil war. I saw a lot of bad stuff.

Have you been the perpetrator of abuse in the past? If so please provide details and dates.
None

Medical - BPSA

Do you have any medical problems? Including dental problems

☐ Yes ☒ No

If yes, please explain

Are you currently under the care of a medical doctor?

☒ Yes ☐ No

Please provide medical doctors name and when your last physical exam was

Sandra Lee, CRNP I saw her about 3 weeks ago.

Do you have any life-threatening allergies?

☐ Yes ☒ No

Explain adverse reactions. Do you have a rescue medication with you?

Do you have any communicable disease(s)?

☐ Yes ☒ No

Explain:

Do you have any chronic medical conditions?

☐ Yes ☒ No

Condition	Date of Onset	Current treatment Provider	Current intervention and response to intervention

MEDICATION WILL BE FOUND IN THE RX BUBBLE ON THE CLIENT CHART

Have you had any major injuries or surgeries?

☐ Yes ☒ No

Date	Location	Reason	Outcome

Date	Location	Reason	Outcome

Do you have any food allergies? Explain adverse reactions.
None

Do you have any drug allergies? Explain adverse reactions.
None

Educational Level - BPSA

Highest Grade/Degree Completed:
College Drop out outside of US

Do you have any diagnosed learning disabilities including attention deficit hyperactivity disorders?
☐ Yes ☒ No

Explain:

Does this affect you today?
☐ Yes ☒ No

How?

Do you have current academic goals?
no

Vocational Screen - BPSA

Are you currently employed?
☒ Yes ☐ No

Explain:
Warehouse employee- full time CNH.

Do you need assistance developing job skills or find a job?
☐ Yes ☒ No

Have you ever missed work or lost a job due to your substance use?
☐ Yes ☒ No

Explain:

What is your current financial status?

☐ Self Sufficient ☐ Supported by Family ☒ Struggling

Military History - BPSA

Have you ever served in the military?

☐ Yes ☒ No

If yes, which branch?

Dates served:

Recreation/ Leisure Activities - BPSA

What do you do for social or leisure activities and how can it support your recovery? Pleast list any current support groups or social services that are currently being utilized.

For a few years now I don't have any fun.

Describe how substance use has impacted this area:

Alcohol was the thing I used to relax and de-stress.

Legal History - BPSA

History of Legal Problems?

☒ Yes ☐ No

Date	Charge	Outcome	State
2018-06-23	DUI	ARD	PA
2022-06-23	DUI	Fines and probation. House arrest	PA
2026-02-14	DUI	Pending	PA

Do you have any criminal charges pending?

☒ Yes ☐ No

Explain:

DUI. Last court date was March 26th. Everything is still pending.

Are you currently on parole or probation?

☒ Yes ☐ No

What agency?

Lancaster County

Do you have a valid driver's license?

☒ Yes ☐ No

Do you have transportation needs?

☐ Yes ☒ No

Explain:

Gender/ Sexual Orientation and sexual functioning - BPSA

Orientation

Heterosexual

Have you put yourself in any risky sexual situations?

☐ Yes ☒ No

If yes, Please explain

Do you have any guilt or shame regarding your sexual orientation or sexual behavior?

None

Spiritual/ Religious Assessment - BPSA

Religious/ Spiritual Preference:

Raised Hindu

Practicing?

☐ Yes ☒ No

What is your history with spirituality?

I was raised hindu.

What kind of spiritual activities do you participate in and how does it affect your recovery?

I don't really practice anymore. I do some stuff.

Culture - BPSA

Ethnic background, cultural beliefs and practices:
None

SNAG/SNAP

Strengths : I am a hard worker. I care about my family a lot.
Needs : I feel like I am lost for now. I used to be good at everything but now I feel lost.
Abilities : I don't know I just feel like my skills are gone now because of my legal issues.
Goals (Preferences) : I want to get out of the legal issue and I want to quit alcohol forever.

Generalized Anxiety Disorder 7-item (GAD-7) Scale

GAD-7

Over the last 2 weeks, how often have you been bothered by the following problems?

1. Feeling nervous, anxious or on edge

☒ Not at all

☐ Several days

☐ More than half the days

☐ Nearly every day
2. Not being able to stop or control worrying

☐ Not at all

☒ Several days

☐ More than half the days

☐ Nearly every day
3. Worrying too much about different things

☐ Not at all

☐ Several days

☐ More than half the days

☒ Nearly every day
4. Trouble relaxing

☐ Not at all

☒ Several days

☐ More than half the days

☐ Nearly every day
5. Being so restless that it is hard to sit still

☒ Not at all

☐ Several days

☐ More than half the days

☐ Nearly every day
6. Becoming easily annoyed or irritable

☒ Not at all

☐ Several days

☐ More than half the days

☐ Nearly every day
7. Feeling afraid as if something awful might happen

☒ Not at all

☐ Several days

☐ More than half the days

☐ Nearly every day

GAD7 Score:
5

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?
☐ Not difficult at all ☒ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Patient Health Questionnaire (PHQ-9)

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

1. Little interest or pleasure in doing things.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

3. Trouble falling asleep, staying asleep, or sleeping too much.

- ☐ Not at all ☐ Several Days ☒ More than half the days ☐ Nearly every day

4. Feeling tired or having little energy.

- ☐ Not at all ☒ Several Days ☐ More than half the days ☐ Nearly every day

5. Poor appetite or overeating.

- ☐ Not at all ☒ Several Days ☐ More than half the days ☐ Nearly every day

6. Feeling bad about yourself - or that you are a failure or have let yourself or your family down

- ☐ Not at all ☐ Several Days ☒ More than half the days ☐ Nearly every day

7. Trouble concentrating on things such as reading the newspaper or watching television.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

8. Moving or speaking so slowly that other people could have noticed? Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

9. Thoughts you would be better off dead or of hurting yourself in some way

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

PHQ-9 Score:

6

10. If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

- ☐ Not difficult at all ☒ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Brief Assessment of Recovery Capital (BARC-10)

Brief Assessment of Recovery Capital (BARC-10)

Instructions: On a scale of 1 (Strongly disagree) to 6 (Strongly agree), please indicate your level of agreement with the following statements.

1. There are important things to me in life than using substances.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☐ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☐ 5 Agree ☒ 6 Strongly Agree

2. In general I am happy with my life.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☐ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☒ 5 Agree ☐ 6 Strongly Agree

3. I have enough energy to complete the tasks I set myself.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☐ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☒ 5 Agree ☐ 6 Strongly Agree

4. I am proud of the community I live in and feel part of it.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☐ 3 Somewhat Disagree ☒ 4 Somewhat Agree ☐ 5 Agree ☐ 6 Strongly Agree

5. I get lots of support from friends.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☒ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☐ 5 Agree ☐ 6 Strongly Agree

6. I regard my life as challenging and fulfilling without the need for using drugs or alcohol.

- ☐ 1 Strongly Disagree ☒ 2 Disagree ☐ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☐ 5 Agree ☐ 6 Strongly Agree

7. My living space has helped to drive my recovery journey.

- ☐ 1 Strongly Disagree ☐ 2 Disagree ☐ 3 Somewhat Disagree ☐ 4 Somewhat Agree ☒ 5 Agree ☐ 6 Strongly Agree

8. I take full responsibility for my actions.

☐ 1 Strongly Disagree

☐ 2 Disagree

☐ 3 Somewhat Disagree

☐ 4 Somewhat Agree

☒ 5 Agree

☐ 6 Strongly Agree

9. I am happy dealing with a range of professional people.

☐ 1 Strongly Disagree☐ 2 Disagree☐ 3 Somewhat Disagree☒ 4 Somewhat Agree☐ 5 Agree☐ 6 Strongly Agree

10. I am making good progress on my recovery journey.

☐ 1 Strongly Disagree☐ 2 Disagree☐ 3 Somewhat Disagree☐ 4 Somewhat Agree☒ 5 Agree☐ 6 Strongly Agree

Scoring

Total scores can range from a miinimm of 10 to a maximum of 60. A ROC Curve Analysis showed the BARC-10 had predictive validity with sustained remission (i.e., 1 year or more) using a cut-off score of 47 in a sample whose average length of recovery time was 7 years.

44

Diagnosis				
Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F10.20] Alcohol use disorder, Severe	Primary	Caleb Madison	04/23/2026	--

Mental Status Summary

Hygiene

☒ WNL(within normal limit)

☐ Odorous

☐ Clean

☐ Disheveled

☐ Unshaved

☐ Other

Comments:

Affect

☒ WNL(within normal limit)

☐ Labile

☐ Restricted

☐ Anxious

☐ Appropriate

☐ Blunted

☐ Expansive

☐ Angry

☐ Inappropriate

☐ Flat

☐ Euphoric

☐ Other

Comments:

Hallucinations

☒ WNL(within normal limit)

☐ Tactile

☐ Denied

☐ Visual

☐ Olfactory

☐ Other

☐ Auditory

☐ Command

Comments:

Movement

☒ WNL(within normal limit)

☐ Fidgety

☐ Psychomotor Agitation

☐ Tics

☐ Normal

☐ Hyperactive

☐ Pacing

☐ Other

☐ Tense

☐ Psychomotor Retardation

☐ Hand Wringing

Comments:

Mood

☒ WNL(within normal limit)

☐ Depressed

☐ Euphoric

☐ Irritated

☐ Other

☐ Anxious

☐ Labile

☐ Angry

☐ Euthymic

☐ Confused

☐ Fearful

☐ Dysthymic

☐ Normal

Comments:

Speech

☒ WNL(within normal limit)

☐ Loud

☐ Slowed

☐ Normal

☐ Pressured

☐ Slurred

☐ Soft

☐ Rapid

☐ Incoherent

Comments:

Impulse Control

- ☒ WNL(within normal limit)
- ☐ Poor

☐ Good

☐ Limited

Comments:

Insight

- ☒ WNL(within normal limit)
- ☐ Poor

☐ Good

☐ Limited

Comments:

Intelligence

- ☒ WNL(within normal limit)
- ☐ Below Average

☐ Above Average

☐ Average

Comments:

Oriented

- ☒ WNL(within normal limit)
- ☐ Below Average

☐ Person

☐ Situation

☐ Place

Comments:

Other

- ☒ WNL(within normal limit)
- ☐ Depersonalization

☐ Obsessions

☐ DeRealism

☐ Compulsions

☐ Denied

Comments:

Memory

- ☒ WNL(within normal limit)
- ☐ ShortTermImpaired

☐ Intact

☐ LongTermImpaired

☐ ImmediateImpaired

Comments:

Suicidal Ideation

- ☒ WNL(within normal limit)
- ☐ Intent

☐ Yes

☐ Means

☐ Plan

☐ Denied

Comments:

Homicidal Ideation

- ☒ WNL(within normal limit)
- ☐ Intent

☐ Yes

☐ Means

☐ Plan

☐ Denied

Comments:

Eye Contact

- ☒ WNL(within normal limit)
- ☐ Fleeting

☐ Good

☐ Stare

☐ Avoidant

Comments:

Facial Expression

- ☒ WNL(within normal limit)
- ☐ Angry
- ☐ Sad

☐ Amiable

☐ Worried

☐ Happy

☐ Suspicious

Comments:

Thought Process

- ☒ WNL(within normal limit)
- ☐ Loose
- ☐ Flight

☐ Intact

☐ Incoherent

☐ Circumstantial

☐ Blocked

☐ Tangential

☐ Other

Comments:

Delusions

- ☒ WNL(within normal limit)
- ☐ Somatic
- ☐ Denied

☐ Grandiose

☐ Paranoia

☐ Other

☐ Persecutory

☐ Ideas

Comments:

Demeanor

☒ WNL(within normal limit)
☐ Guarded
☐ Friendly
☐ Demanding
☐ Worried
☐ Seductive
☐ Other

☐ Cooperative
☐ Resistive
☐ Worried
☐ Friendly
☐ Evasive
☐ Arrogant

☐ Uncooperative
☐ Defensive
☐ Boastful
☐ Irritable
☐ Dramatic
☐ Apathetic

Comments:

Attention/Concentration

☒ WNL(within normal limit)
☐ Easily Distracted

☐ Satisfactory
☐ Other

☐ Poor

Comments:

Judgment

☒ WNL(within normal limit)
☐ Poor

☐ Good
☐ Other

☐ Limited

Comments:

ASAM Placement

ASAM Placement Summary Sheet

Date:

2026-04-23

Type (Check One):

☒ Admission ☐ Continued Stay ☐ Discharge

Dimension

	Level of Risk	Level of Care	Criteria Included/Comments
D1. Acute intoxication and/or withdrawal potential:	0	2.5	denies current alcohol use, with last reported use on 02/14/2026. He is currently receiving Vivitrol, which he reports has been effective in reducing cravings. There are no reported symptoms of acute intoxication or withdrawal, and no history of severe withdrawal complications such as seizures or delirium tremens. At this time, he presents as medically stable with minimal risk in this dimension.
D2. Biomedical Conditions and Complications:	0	2.5	denies any significant medical conditions, chronic illnesses, or acute biomedical concerns. He reports being under the care of a medical provider and having had a recent physical exam approximately three weeks prior to intake. There are no reported medical issues that would interfere with treatment participation. He is considered stable in this dimension.

	Level of Risk	Level of Care	Criteria Included/Comments
D3. Emotional/Behavioral or cognitive conditions and complications:	2	2.5	█████ reports a history of depression and is currently prescribed Prozac. He denies current significant symptoms impacting daily functioning, and screening tools indicate mild depression (PHQ-9 score of 6) and mild anxiety (GAD-7 score of 5). He reports a prior involuntary psychiatric hospitalization (302) in February after making statements while intoxicated but denies any true suicidal intent and denies current suicidal or homicidal ideation. While generally stable, his history of depression, trauma exposure, and emotionally driven drinking patterns indicate some vulnerability, elevating risk to low-moderate.
D4. Readiness to change:	3	2.5	█████ demonstrates ambivalence regarding his substance use. While he acknowledges the significant legal consequences of his drinking, including multiple DUIs, he has not fully accepted a diagnosis of alcoholism. He stated, "I am not an alcoholic, but when you get 4 DUIs no one will believe you," indicating external awareness of consequences without full internal acceptance of the severity of his condition. His motivation for treatment appears largely externally driven by legal pressures rather than intrinsic commitment to change. This ambivalence significantly increases his risk for disengagement and relapse without continued therapeutic intervention focused on insight development and motivational enhancement.
D5. Relapse, continued use or continued problem potential:	3	2.5	█████ presents with a significant history of relapse, including a return to use following an 18-month period of sobriety. His relapse was associated with attempts at controlled drinking, which quickly escalated. He identifies emotional distress, frustration, and life stressors as primary triggers but demonstrates limited coping strategies outside of substance use. His history of binge drinking, repeated legal consequences, and difficulty maintaining long-term sobriety indicate a high risk of continued use without structured intervention. Although Vivitrol is currently helping with cravings, his lack of internalized recovery principles and coping mechanisms further elevate relapse risk.
D6. Recovery Environment:	2	2.5	█████ reports a generally supportive home environment, including a spouse who encourages sobriety and maintains a substance-free household. However, his broader recovery environment is limited. He reports minimal engagement in sober social supports, no current involvement in recovery groups, and limited recreational or leisure activities. Family relationships have been negatively impacted by his substance use, particularly with his child. While the home environment is a protective factor, the lack of a robust recovery network and limited engagement in pro-recovery activities increases his overall risk.

Indicate level of care recommended:

2.5

Indicate level of care received:

2.1

If recommended level of care is different from received, why?

While PHP is clinically indicated, [REDACTED]'s current level of insight and readiness for change suggests he may not fully engage in a higher level of care at this time. Additionally, he maintains full-time employment and has family responsibilities that would be significantly disrupted by PHP participation. In order to support engagement and retention in treatment, an evening IOP is recommended as a more realistic and appropriate level of care. This level will allow him to maintain employment while still receiving structured therapeutic support, psychoeducation, and relapse prevention services. Treatment will focus on increasing motivation for change, developing insight into substance use disorder, building coping skills for emotional regulation, and establishing a stronger recovery support network.

Indicate the program or Facility referred to:
R3 Recovery Services

Narrative Summary

Narrative Summary

[REDACTED] Singh Chuwan is a 40-year-old male who presented for a biopsychosocial assessment following his third DUI, which is currently pending and has resulted in court-mandated evaluation and treatment. He identifies alcohol as his primary substance of use and reports a long-standing pattern of binge drinking that began at age 20. His most recent use occurred on 02/14/2026. [REDACTED] reports that he is currently receiving Vivitrol, which he states has been effective in reducing cravings and supporting his current abstinence.

[REDACTED] has a history of prior sobriety, with his longest period lasting approximately 18 months following his second DUI. During that time, he attributes his success largely to external accountability, specifically the use of an interlock device. He reports that his relapse occurred when he attempted to reintroduce alcohol in a controlled manner, limiting his drinking to his home environment; however, this quickly escalated back into problematic use. [REDACTED] demonstrates moderate insight into his relapse patterns, identifying emotional distress, frustration, and life stressors as primary triggers that increase his urge to drink. He acknowledges that alcohol has been his primary method of coping with stress and reports limited engagement in alternative coping strategies or leisure activities, noting that he has not experienced enjoyment or recreational fulfillment in recent years.

[REDACTED] reports significant consequences related to his substance use, most notably legal issues including multiple DUIs (2018, 2022, and 2026), with the most recent charge still pending. He is currently on probation through Lancaster County. He also reports interpersonal consequences, particularly within his family. [REDACTED] is married and has children, and he describes his wife as supportive, noting that there is no alcohol allowed in the home. However, his alcohol use has created strain, particularly with his son, who reportedly avoids communication with him when he is drinking. [REDACTED] also shared a significant emotional stressor related to past concerns about paternity, which contributed to periods of increased alcohol use and relapse. Despite these challenges, he identifies his wife as a primary source of support and expresses a strong desire to improve his family relationships.

[REDACTED] reports a history of depression and is currently prescribed Prozac, though he denies any current significant mental health symptoms impacting his functioning. He does report a prior involuntary psychiatric hospitalization (302) in February after making statements while intoxicated, though he denies any true intent to harm himself at that time and denies any current suicidal or homicidal ideation. His mental status exam is within normal limits across all domains, including mood, affect, thought process, insight, judgment, and orientation. Screening tools indicate mild symptoms of anxiety (GAD-7 score of 5) and depression (PHQ-9 score of 6).

[REDACTED]'s psychosocial history is notable for early life instability, including his father leaving the family when he was seven years old and being raised by a single mother in Nepal during a time of civil war, which he reports exposed him to significant trauma. He later immigrated to the United States in 2010. He describes himself as having been a well-behaved and academically capable child. Currently, he reports stable employment as a full-time warehouse worker and identifies as self-sufficient financially. However, he expresses a sense of lost identity and diminished self-efficacy, stating that he previously felt competent but now feels "lost" due to his legal and substance use issues.

[REDACTED] identifies several strengths that may support his recovery, including being a hard worker, caring deeply for his family, and expressing a clear desire to resolve his legal issues and achieve long-term sobriety. At the same time, he demonstrates limited internal coping skills and continues to rely heavily on external motivators for change. His recovery environment appears supportive overall, with a substance-free home and encouragement from his spouse, though his broader social support network appears limited. He is not currently engaged in recovery support groups and reports minimal participation in structured recreational or social activities.

Overall, [REDACTED] presents with Alcohol Use Disorder, Severe, characterized by a chronic pattern of binge drinking, repeated legal consequences, relapse following attempts at controlled use, and continued use despite interpersonal and occupational impacts. While he demonstrates motivation for change, much of this appears externally driven by legal consequences, though there is emerging internal motivation related to family and quality of life. His clinical presentation suggests a need for structured treatment focused on relapse prevention, development of coping skills, emotional regulation, and strengthening of recovery supports to promote sustained sobriety and reduce risk of future legal involvement.

Digitally Signed by



Caleb Madison, BS 04/24/2026 10:28AM

Reviewer [REDACTED]



----- END OF BPS INTAKE -----

INTAKE: MEDICAL - HISTORY & PHYSICAL

Chief Complaint

Present Illness History

Drug Of Choice

Substance Use History: --
Drug of Choice 1: --
Drug of Choice 2: --
Drug of Choice 3: --
Additional substance use history (include withdrawal history and medical complications): --

Mental Health History

Have you ever been given a psychiatric diagnosis? ☐
Have you received mental health services or are you currently receiving services? ☐
Comments : N/A

Past Surgical and Hospitalization History List

-- No Past Surgical or Hospitalization History --

Allergies

-- No Allergies --

Lab Results

Radiology Reports

Assessment

--

Initial Treatment Plan

Medication Orders

Consult Orders

----- END OF MEDICAL INTAKE -----

INTAKE: MEDICAL - NURSING

Vital Signs

--No Vitals --

Allergies

-- No Allergies --

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Alcohol	0	Drinking	Unknown I would binge drink.	02/14/2026	20	9	buy it.	Binge

Mental Status Summary

Hygiene

- ☒ WNL(within normal limit)
- ☐ Odorous

- ☐ Clean
- ☐ Disheveled

- ☐ Unshaved
- ☐ Other

Comments:

Affect

- ☒ WNL(within normal limit)
- ☐ Labile
- ☐ Restricted
- ☐ Anxious

- ☐ Appropriate
- ☐ Blunted
- ☐ Expansive
- ☐ Angry

- ☐ Inappropriate
- ☐ Flat
- ☐ Euphoric
- ☐ Other

Comments:

Hallucinations

- ☒ WNL(within normal limit)
- ☐ Tactile
- ☐ Denied

- ☐ Visual
- ☐ Olfactory
- ☐ Other

- ☐ Auditory
- ☐ Command

Comments:

Movement

- ☒ WNL(within normal limit)
- ☐ Fidgety
- ☐ Psychomotor Agitation
- ☐ Tics

- ☐ Normal
- ☐ Hyperactive
- ☐ Pacing
- ☐ Other

- ☐ Tense
- ☐ Psychomotor Retardation
- ☐ Hand Wringing

Comments:

Mood

- ☒ WNL(within normal limit)
- ☐ Depressed
- ☐ Euphoric
- ☐ Irritated
- ☐ Other

- ☐ Anxious
- ☐ Labile
- ☐ Angry
- ☐ Euthymic

- ☐ Confused
- ☐ Fearful
- ☐ Dysthymic
- ☐ Normal

Comments:

Speech

- ☒ WNL(within normal limit)
- ☐ Loud
- ☐ Slowed

- ☐ Normal
- ☐ Pressured
- ☐ Slurred

- ☐ Soft
- ☐ Rapid
- ☐ Incoherent

Comments:

Impulse Control

- ☒ WNL(within normal limit)
- ☐ Poor

- ☐ Good

- ☐ Limited

Comments:

Insight

- ☒ WNL(within normal limit)
- ☐ Poor

☐ Good

☐ Limited

Comments:

Intelligence

- ☒ WNL(within normal limit)
- ☐ Below Average

☐ Above Average

☐ Average

Comments:

Oriented

- ☒ WNL(within normal limit)
- ☐ Below Average

☐ Person

☐ Situation

☐ Place

Comments:

Other

- ☒ WNL(within normal limit)
- ☐ Depersonalization

☐ Obsessions

☐ DeRealism

☐ Compulsions

☐ Denied

Comments:

Memory

- ☒ WNL(within normal limit)
- ☐ ShortTermImpaired

☐ Intact

☐ LongTermImpaired

☐ ImmediateImpaired

Comments:

Suicidal Ideation

- ☒ WNL(within normal limit)
- ☐ Intent

☐ Yes

☐ Means

☐ Plan

☐ Denied

Comments:

Homicidal Ideation

- ☒ WNL(within normal limit)
- ☐ Intent

☐ Yes

☐ Means

☐ Plan

☐ Denied

Comments:

Eye Contact

- ☒ WNL(within normal limit)
- ☐ Fleeting

☐ Good

☐ Stare

☐ Avoidant

Comments:

Facial Expression

- ☒ WNL(within normal limit)
- ☐ Angry
- ☐ Sad

☐ Amiable

☐ Worried

☐ Happy

☐ Suspicious

Comments:

Thought Process

- ☒ WNL(within normal limit)
- ☐ Loose
- ☐ Flight

☐ Intact

☐ Incoherent

☐ Circumstantial

☐ Blocked

☐ Tangential

☐ Other

Comments:

Delusions

- ☒ WNL(within normal limit)
- ☐ Somatic
- ☐ Denied

☐ Grandiose

☐ Paranoia

☐ Other

☐ Persecutory

☐ Ideas

Comments:

Demeanor

- ☒ WNL(within normal limit)
- ☐ Guarded
- ☐ Friendly
- ☐ Demanding

☐ Cooperative

☐ Resistive

☐ Worried

☐ Friendly

☐ Uncooperative

☐ Defensive

☐ Boastful

☐ Irritable

- ☐ Worried
- ☐ Seductive
- ☐ Other
- ☐ Evasive
- ☐ Arrogant
- ☐ Dramatic
- ☐ Apathetic

Comments:

Attention/Concentration

- ☒ WNL(within normal limit)
- ☐ Easily Distracted
- ☐ Satisfactory
- ☐ Other
- ☐ Poor

Comments:

Judgment

- ☒ WNL(within normal limit)
- ☐ Poor
- ☐ Good
- ☐ Other
- ☐ Limited

Comments:

----- END OF NURSING INTAKE -----

INTAKE: MEDICAL - PSYCHIATRIC EVALUATION

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Alcohol	0	Drinking	Unknown I would binge drink.	02/14/2026	20	9	buy it.	Binge

COWS Scores

COWS Date	COWS Time	Total COWS Score	Scale Score	Created By	Created Date
-- No COWs --					

CIWA Scores

CIWA Date	CIWA Time	Total CIWA Score	Created By	Created Date
-- No CIWAs --				

CIWA-B Scores

CIWA-B Date	CIWA-B Time	Total CIWA-B Score	Created By	Created Date
-- No CIWA-Bs --				

----- END OF MEDICAL INTAKE -----

INTAKE: BILLING

Insurance

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care	Singh Chuwan	Self	None Specified	771216145812	Primary		Active

Private Pay

-- No Private Pay --

----- END OF BILLING INTAKE -----

TREATMENT PLAN

Treatment Plan: (Initial TP)
04/23/2026 - 04/23/2027

Primary Clinician: **Marleigh Johnson**

Expected Length of Treatment
(days): **365**

Problem: Substance Use Disorders

Diagnostic Impression:

04/23/2026 - [F10.20] Alcohol use disorder, Severe

Behavioral Definitions:

04/23/2026 - Continues substance use despite knowledge of experiencing persistent physical, legal, financial, vocational, social, and/or relationship problems that are directly caused by the use of the substance.

Client Goal: Client will increase understanding of addiction, treatment expectations, and program structure to support engagement and successful participation in treatment.

Objective: Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Intervention: Therapist will support the development of therapeutic rapport by creating a safe, nonjudgmental space for open communication, encouraging client engagement, and facilitating introductions to peers and staff to promote connection within the treatment community.

05/23/2026: Status - (Open)

Objective: Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Intervention: Primary therapist will provide psychoeducation on addiction, recovery concepts, and program expectations through individual sessions and group facilitation to increase client's knowledge and insight into their substance use and treatment process.

05/23/2026: Status - (Open)

Treatment Plan Addendum

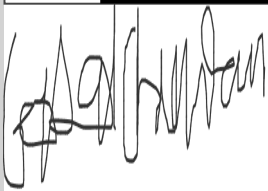
Level of Care:

- ☐ PHP (2.5)
- ☐ IOP 19 (2.1)
- ☐ IOP 5 (2.1)
- ☒ IOP 3 (2.1)
- ☐ OP (1.0)

Treatment Interventions to Support Established Treatment Plan:

- ☐ Individual Sessions
 - ☒ 1x per week
 - ☐ 2x per week
 - ☐ Other:
- ☐ Group Sessions
 - ☐ 5x per week
 - ☐ 4x per week
 - ☒ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:

Client



Chuwan

April 23, 2026 03:41 PM

Staff



Caleb Madison, BS

April 23, 2026 03:41 PM

Reviewer

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC
April 23, 2026 03:41 PM

Treatment Plan: 05/05/2026 - 05/05/2027	Primary Clinician: Marleigh Johnson	Expected Length of Treatment (days): 365
---	-------------------------------------	--

Problem: Substance Use

Diagnostic Impression:
05/05/2026 - [F10.20] Alcohol use disorder, Severe

Behavioral Definitions:
05/05/2026 - [REDACTED] demonstrates a pattern of substance use that has resulted in significant legal consequences, as evidenced by three DUI offenses. His continued use despite these consequences indicates impaired judgment, decreased impulse control, and difficulty utilizing effective coping strategies. Substance use appears to contribute to high-risk behaviors, including driving while under the influence, placing himself and others at risk. These behaviors are consistent with a substance use disorder and reflect the need for structured intervention to support sobriety, improve decision-making, and reduce the likelihood of future legal involvement.

Client Goal: [REDACTED] will maintain sobriety by increasing insight into triggers, strengthening relapse prevention strategies, and consistently utilizing coping skills through participation in individual and group therapy sessions.

Objective: [REDACTED] will demonstrate use of at least 3 coping skills (e.g., journaling, peer support, urge surfing) to manage cravings, as evidenced by self-report and group participation over the next 60 days.

Intervention: Therapist will utilize CBT and relapse prevention strategies in both individual and group sessions to help [REDACTED] identify triggers, increase insight, and develop coping responses.

07/04/2026: Status - (Open)

Objective: [REDACTED] will identify at least 3 personal triggers for substance use and process them in individual and group sessions within 30 days.

Intervention: Therapist will provide psychoeducation and facilitate skill-building exercises in group and individual sessions to strengthen [REDACTED]'s ability to manage cravings and maintain sobriety.

07/04/2026: Status - (Open)

Client Goal: [REDACTED] will improve overall stability by developing financial independence and securing consistent, safe living arrangements through skill-building and engagement in individual and group therapy sessions.

Objective: [REDACTED] will explore and identify at least 2 realistic options for improving financial stability (e.g., employment, budgeting strategies) within 45 days.

Intervention: Therapist will support [REDACTED] in identifying financial goals, reviewing resources, and building problem-solving skills through structured discussion in individual sessions and peer feedback in group sessions.

07/04/2026: Status - (Open)

Objective: [REDACTED] will identify and initiate at least 1 step toward improving living stability (e.g., securing housing, creating a structured daily routine) within 60 days.

Intervention: Therapist will assist [REDACTED] in developing a structured plan to enhance living stability, including referrals to community resources and accountability check-ins during individual and group sessions.

07/04/2026: Status - (Open)

Treatment Plan Addendum

Level of Care:

- ☐ PHP (2.5)
- ☐ IOP 19 (2.1)
- ☐ IOP 5 (2.1)
- ☒ IOP 3 (2.1)
- ☐ OP (1.0)

Treatment Interventions to Support Established Treatment Plan:

- ☒ Individual Sessions
 - ☒ 1x per week
 - ☐ 2x per week
 - ☐ Other:
- ☒ Group Sessions
 - ☐ 5x per week
 - ☐ 4x per week
 - ☒ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:

Client [REDACTED]

Gregory Johnson

 Chuwan
May 05, 2026 02:45 PM

Staff 


Marleigh Johnson, MS, LPC
May 05, 2026 02:45 PM

----- END OF TX PLAN -----

TREATMENT PLAN REVIEW

-- No Treatment Plan Reviews --

----- END OF TX PLAN REVIEW -----

DISCHARGE / TRANSFER

ClientName: [REDACTED] Chuwan	Age: 40 Years	Date of Birth: 12/31/1985
Admit Date: 04/23/2026	Length Of Stay: 55 days	Discharge Date: Apr 23 2026 12:00AM

Master Discharge/Transfer Overview

Version	Author	Created Date
MDP_MDP_Apr 23 2026 3:26PM (1)	Caleb Madison, BS	04/23/2026 3:26PM

Reason for Admission:

Explanation For Changes in Diagnosis, if any:

Strengths:

I am a hard worker. I care about my family a lot.

Needs:

I feel like I am lost for now. I used to be good at everything but now I feel lost.

Abilities:

I don't know I just feel like my skills are gone now because of my legal issues.

Goals (Preferences):

I want to get out of the legal issue and I want to quit alcohol forever.

Anticipated Discharge Date:

04/23/2026

Reason for Discharge:

Currently In Treatment

Discharge Transition Obstacles:

Family/Guardian Participation in Treatment:

Critical Events & Interaction:

Prognosis:

Living Arrangements:

Services Provided:

Recommendations:

Summary of Treatment Goals:

Active Meds:

Drug	Instruction
Prozac 20 mg capsule	Take 20 mg by mouth
Vivitrol 380 mg intramuscular suspension,extended release	Take 380 mg intramuscular

Diagnosis:

Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F10.20] Alcohol use disorder, Severe	Primary	Caleb Madison	04/23/2026	--

Staff

[REDACTED]



Caleb Madison, BS
Apr 23 2026 4:26PM -04:00

----- END OF DISCHARGE / TRANSFER -----

DISCHARGE SUMMARY

ClientName: [REDACTED] Chuwan	Age: 40 Years	Date of Birth: 12/31/1985
Admit Date: Apr 23 2026	Length Of Stay: 55 days	Discharge Date: N/A

----- END OF DISCHARGE SUMMARY -----

OTHER NOTES

Note By	Note	Note Type
04/24/2026		
Caleb Madison, BS	@New Admit @Marleigh Johnson, MS, LPC This member will be in IOP-3. He should expect to do it longer than the normal step down as he came in at a lower level of care then was recommended. ██████ Singh Chuwan is a 40-year-old male who presented for a biopsychosocial assessment following his third DUI, which is currently pending and has resulted in court-mandated evaluation and treatment. He identifies alcohol as his primary substance of use and reports a long-standing pattern of binge drinking that began at age 20. His most recent use occurred on 02/14/2026. ██████ reports that he is currently receiving Vivitrol, which he states has been effective in reducing cravings and supporting his current abstinence. ██████ has a history of prior sobriety, with his longest period lasting approximately 18 months following his second DUI. During that time, he attributes his success largely to external accountability, specifically the use of an interlock device. He reports that his relapse occurred when he attempted to reintroduce alcohol in a controlled manner, limiting his drinking to his home environment; however, this quickly escalated back into problematic use. ██████ demonstrates moderate insight into his relapse patterns, identifying emotional distress, frustration, and life stressors as primary triggers that increase his urge to drink. He acknowledges that alcohol has been his primary method of coping with stress and reports limited engagement in alternative coping strategies or leisure activities, noting that he has not experienced enjoyment or recreational fulfillment in recent years. ██████ reports significant consequences related to his substance use, most notably legal issues including multiple DUIs (2018, 2022, and 2026), with the most recent charge still pending. He is currently on probation through Lancaster County. He also reports interpersonal consequences, particularly within his family. ██████ is married and has children, and he describes his wife as supportive, noting that there is no alcohol allowed in the home. However, his alcohol use has created strain, particularly with his son, who reportedly avoids communication with him when he is drinking. ██████ also shared a significant emotional stressor related to past concerns about paternity, which contributed to periods of increased alcohol use and relapse. Despite these challenges, he identifies his wife as a primary source of support and expresses a strong desire to improve his family relationships. ██████ reports a history of depression and is currently prescribed Prozac, though he denies any current significant mental health symptoms impacting his functioning. He does report a prior involuntary psychiatric hospitalization (302) in February after making statements while intoxicated, though he denies any true intent to harm himself at that time and denies any current suicidal or homicidal ideation. His mental status exam is within normal limits across all domains, including mood, affect, thought process, insight, judgment, and orientation. Screening tools indicate mild symptoms of anxiety (GAD-7 score of 5) and depression (PHQ-9 score of 6). ██████s psychosocial history is notable for early life instability, including his father leaving the family when he was seven years old and being raised by a single mother in Nepal during a time of civil war, which he reports exposed him to significant trauma. He later immigrated to the United States in 2010. He describes himself as having been a well-behaved and academically capable child. Currently, he reports stable employment as a full-time warehouse worker and identifies as self-sufficient financially. However, he expresses a sense of lost identity and diminished self-efficacy, stating that he previously felt competent but now feels “lost” due to his legal and substance use issues. ██████ identifies several strengths that may support his recovery, including being a hard worker, caring deeply for his family, and expressing a clear desire to resolve his legal issues and achieve long-term sobriety. At the same time, he demonstrates limited internal coping skills and continues to rely heavily on external motivators for change. His recovery environment appears supportive overall, with a substance-free home and encouragement from his spouse, though his broader social support network appears limited. He is not currently engaged in recovery support groups and reports minimal participation in structured recreational or social activities. Overall, ██████ presents with Alcohol Use Disorder, Severe, characterized by a chronic pattern of binge drinking, repeated legal consequences, relapse following attempts at controlled use, and continued use despite interpersonal and occupational impacts. While he demonstrates motivation for change, much of this appears externally driven by legal consequences, though there is emerging internal motivation related to family and quality of life. His clinical presentation suggests a need for structured treatment focused on relapse prevention, development of coping skills, emotional regulation, and strengthening of recovery supports to promote sustained sobriety and reduce risk of future legal involvement.	Informational
04/30/2026		
Marleigh Johnson, MS, LPC	@Schedule Changes ██████ states that he can not make his session time work. Please contact him to find a session time in my hours that will work for him. If he can't find time that works on my schedule he will have to be discharged.	Informational
05/06/2026		
Marleigh Johnson, MS, LPC	@Schedule Changes Please put ██████ on my schedule for next week on Monday at 11am just for next week. After that, please add him to my schedule for monday at 7:30 am ongoing.	Informational
05/12/2026		
Melissa Reed, AA	@Marleigh Johnson, MS, LPC ██████ wanted me to let you know that he has court tomorrow, and his lawyer advised him that he could be spending 2 days in jail. He stated that he will not be able to contact us to let us know if he will not be available for group, tomorrow- but that he'd be back on Thursday if all goes as expected. Thank you!	Informational
06/12/2026		
Marleigh Johnson, MS, LPC	@Schedule Changes @LOC Changes ██████ is moving to OP effective 5/13. He will do Monday night group and keep his monday morning sessions.	Informational

